NCE EXAM PREP

Practice Exam 01

170 Practice Questions

225 minutes | A, B, C answer choices

Questions followed by answer key and detailed explanations

> **Instructions.** Select the single best answer for every question. Record your answers separately. Work at exam pace, allowing approximately 225 minutes for all 170 questions. The answer key and explanations begin in Part B; avoid viewing them until you have completed Part A.

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PART A — QUESTIONS

Choose one answer (A, B, or C) for each question.

Question 1

When a counselor identifies an apparent conflict between an ethical standard and a controlling law, which response is most appropriate?

A) Follow the ethical standard automatically because professional ethics supersede civil law

B) Ignore the ethical issue after confirming that the law applies to the counselor

C) Clarify the conflict through appropriate consultation, comply with controlling law, and minimize ethical harm

Question 2

Which practice is most likely to keep a counseling program's referral directory usable?

A) Add every known local agency once and preserve the original entry dates for consistency

B) Regularly verify eligibility, services, accessibility, languages, costs, wait times, and contact routes

C) Rank programs only by public online reviews so clients can receive the most popular options

Question 3

Which case-note statement most clearly distinguishes client report from counselor observation?

A) The client appeared severely depressed and probably skipped work throughout the week.

B) The client was evasive about employment and unwilling to improve the situation.

C) The client reported missing three shifts; the counselor observed slowed speech and tearfulness during the discussion.

Question 4

If a condition is very uncommon in the population being assessed, what commonly happens to the positive predictive value of a screening test when other test characteristics remain constant?

A) It increases because false positives become less likely

B) It decreases because a larger proportion of positive results may be false positives

C) It becomes identical to the test's sensitivity

Question 5

> A newly licensed counselor in a community clinic has general experience treating anxiety but has never delivered exposure and response prevention. A client with obsessive-compulsive disorder wants to begin this treatment, and the clinic can provide an experienced supervisor.

Which professional development step is most appropriate before the counselor delivers the treatment?

A) Attend a brief overview webinar and begin the protocol independently

B) Arrange clinical supervision while obtaining structured training in the treatment

C) Ask an experienced colleague for a one-time opinion and then use the full protocol

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Question 6

Which statement is written as a measurable treatment objective rather than a broad goal or counselor intervention?

A) The client will use two coping strategies during at least four work shifts each week for the next month

B) The client will improve emotional regulation and feel more confident at work

C) The counselor will teach diaphragmatic breathing and cognitive reframing during sessions

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Question 7

> A client with posttraumatic symptoms requests immediate, detailed trauma processing. During the past week, the client has had frequent dissociative episodes, cannot reliably use grounding skills, and lacks a safe place to stay at night.

How should these facts affect the initial treatment plan?

A) Begin intensive trauma processing because the client's request establishes readiness

B) Emphasize stabilization and immediate supports before reconsidering intensive trauma processing

C) Exclude trauma-related goals permanently because dissociation makes trauma treatment ineffective

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Question 8

> An adult client meets criteria for generalized anxiety disorder. When the counselor names the diagnosis, the client looks uneasy and says that in the client's family, a mental health label can imply personal weakness.

What is the best approach to reviewing the diagnosis for treatment planning?

A) Avoid discussing the diagnosis further and record that the client rejected it

B) Explain the evidence and purpose in accessible language, invite the client's perspective, and discuss cultural meaning

C) Emphasize that diagnostic terminology is objective and should not be influenced by family beliefs

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Question 9

> A university counselor is trained to administer and interpret brief symptom screeners. The disability services office asks the counselor to conduct a comprehensive neuropsychological battery for a student's accommodation request. The counselor has never used the battery.

What should the counselor do?

A) Learn the battery from its manual and administer it because the purpose is educational

B) Administer the battery but ask a qualified evaluator to confirm only the final diagnosis

C) Limit services to assessments within current competence and arrange consultation or referral to a qualified evaluator

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Question 10

> Before a skills group begins, a prospective member says, “I assume the counselor can guarantee that no member will ever repeat anything said here.”

Which response best supports informed consent for group counseling?

A) Confirm that the counselor can legally guarantee every member's confidentiality outside the group

B) Avoid discussing confidentiality because doing so may discourage participation

C) Explain the counselor's duties and the expectation that members protect privacy, while clarifying that other members' behavior cannot be guaranteed

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Question 11

> A community agency begins serving more Deaf clients. A counselor knows conversational American Sign Language but is not fluent in clinical communication. One new client requests a qualified interpreter and wants to remain at the agency.

Which action best supports professional development and equitable access?

A) Arrange qualified language access, learn effective interpreter collaboration, and avoid presenting conversational skill as clinical fluency

B) Ask the client to bring a family member while the counselor improves sign-language skills through practice

C) Refer all Deaf clients to another agency until the counselor becomes clinically fluent in American Sign Language

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Question 12

A client treated for obsessive-compulsive symptoms has met the behavioral targets for ten weeks and independently manages occasional symptom increases. Both counselor and client had agreed that these indicators would prompt a termination review. The client worries that ending weekly sessions means support will disappear.

Which response best supports an appropriate transition?

A) Continue weekly counseling for an open-ended period to prevent the client's discomfort

B) Terminate at the end of the current session because the criteria have already been met

C) Review the gains, plan a gradual ending if useful, and clarify booster and reentry options

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Question 13

During a couple's intake, one partner asks whether information disclosed in an individual check-in will always be kept secret from the other partner. The clinic uses a written no-secrets policy for couple counseling.

What should the counselor do before services begin?

A) Promise absolute confidentiality for every individual disclosure made by either partner

B) Wait until a private disclosure occurs before explaining how information will be handled

C) Explain the no-secrets policy, its limits, and how it applies to both partners, then obtain informed agreement

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Question 14

A 10-year-old frequently leaves the assigned area during independent schoolwork. The child, caregiver, and school counselor agree that the behavior is most likely when tasks are long. The child enjoys earning drawing time.

Which behavioral management plan is best designed for consistent use across settings?

A) Remove drawing materials whenever the child leaves the area, with each adult deciding how long they remain unavailable

B) Tell the child to show better self-control and review progress whenever another incident occurs

C) Define a brief work interval, reinforce remaining in the area with drawing time, and record the behavior consistently

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Question 15

A counselor's website lists the counselor as a "certified complex trauma specialist." The counselor completed a weekend workshop with a similar title, but the sponsoring organization did not award a certification and the counselor holds no such credential.

What should the counselor do?

A) Keep the title because completion of relevant training is equivalent to certification for advertising purposes

B) Remove the unsupported title and describe the counselor's actual education, licenses, and credentials accurately

C) Keep the title if the website also states that results from counseling cannot be guaranteed

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Question 16

A Deaf client requests a qualified American Sign Language interpreter for intake. The practice's applicable accessibility rule requires effective communication and states that an adult companion should not be used as an interpreter unless the client requests it and the arrangement is appropriate. The client does not want a family member involved.

What should the counselor arrange?

A) Ask the client to bring a relative because the relative already knows the client's history

B) Conduct the intake through written notes even though the client says this will not provide effective communication

C) Provide a qualified interpreter through an accessible, confidentiality-protective arrangement

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Question 17

A client with chronic pain alternates between doing many household tasks on a low-pain day and being unable to function for several days afterward. The client values contributing at home and wants a more sustainable pattern.

Which intervention best matches the client’s goal?

A) Collaboratively establish paced, graded activity with planned rest before symptoms sharply increase.

B) Encourage the client to complete as much as possible on low-pain days to preserve independence.

C) Recommend avoiding household activity until pain is consistently minimal.

Question 18

> Devon receives services at a community agency and uses an augmentative communication device after a traumatic brain injury. The prior session note states that extended open-ended questioning caused fatigue and that Devon preferred a brief written rating scale with additional response time. The next session will continue work on Devon's goal of increasing independent community activities.

What is the most appropriate way for the counselor to prepare for the session?

A) Prepare the rating scale, allow additional response time, and review the prior goal and fatigue pattern

B) Plan the same interview format used with all clients so Devon is not treated differently

C) Ask Devon's caregiver to answer the questions in advance because Devon's responses take longer

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Question 19

> A recent immigrant seeks counseling for sleep disruption, headaches, and what the client calls “too many nerves” following exposure to community violence. The client does not initially use the word trauma and is unfamiliar with behavioral health screening.

Which assessment approach is most appropriate?

A) Explore the client's meaning of the symptoms, explain the purpose of screening, and use a suitable trauma measure alongside the interview

B) Interpret the phrase “too many nerves” as evidence of generalized anxiety and omit trauma questions to avoid discomfort

C) Use only the counselor's observations because standardized screening is inappropriate whenever a client uses culture-specific language

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Question 20

> A new telehealth client joins an intake from an account bearing a different name and declines to state the current physical location. The practice requires identity, location, emergency-contact, and privacy checks before clinical services begin.

What should the counselor do?

A) Complete the required verification and emergency checks before proceeding with clinical services

B) Begin the intake and collect the information only if a crisis topic arises

C) Rely on the account profile because platform authentication replaces clinical verification

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Question 21

A client entered counseling for panic symptoms that interfered with commuting. At the eighth-week review, the client is commuting independently, uses coping skills without prompting, and has met the agreed symptom and functioning targets. The client is unsure whether counseling should end.

What is the most appropriate way to determine readiness for termination?

A) Continue weekly sessions until the client reports no anxiety in any situation

B) End services immediately because the original behavioral target has been met

C) Review the agreed criteria and remaining needs with the client, then decide on a taper or termination plan together

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Question 22

A counselor in a rural clinic has general experience with adolescents but limited training in eating disorders. A 16-year-old reports restrictive eating, recent fainting, and rapid weight loss. The nearest specialty program can provide medical and behavioral evaluation by telehealth and a regional partner.

Which response best reflects awareness of professional limitations?

A) Begin weekly general counseling and reconsider referral only if eating behavior does not improve

B) Facilitate prompt specialized and medical evaluation and participate in ongoing care only within demonstrated competence

C) Terminate all involvement because any continuing role would exceed the counselor's competence

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Question 23

Before a telehealth session, a counselor verifies the client's current location and emergency contact as required by the practice plan. During the session, the client reports intent to use a loaded firearm immediately and then the video connection fails. Repeated calls to the client do not connect.

What should the counselor do next?

A) Activate the emergency protocol using the verified location and disclose only information needed for the response

B) Wait for the client to reconnect because contacting responders would violate telehealth confidentiality

C) Send a portal message asking the client to schedule an in-person risk assessment the following day

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Question 24

> In the third session of a new interpersonal group, one member speaks at length whenever silence occurs. Several members have stopped trying to enter the conversation and exchange frustrated looks.

What is the leader’s best intervention?

A) Respectfully interrupt, describe the participation pattern, and invite the group to examine how it is affecting them.

B) Allow the member to continue because early group cohesion depends on avoiding direct feedback.

C) Establish a fixed time limit for every member without discussing the reactions already occurring in the group.

Question 25

> A family requests counseling and brings a bilingual 12-year-old to interpret for a parent who has limited English proficiency. The family says using the child will save time, but the parent appears hesitant when sensitive topics are mentioned.

What is the best intake arrangement?

A) Use the child as interpreter because the family has already selected that role

B) Ask only yes-or-no questions so no interpreter is needed

C) Offer a qualified interpreter, explain interpreter confidentiality and roles, and obtain the parent's agreement about communication

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Question 26

> A counselor plans to expand telehealth services across state lines and learns that requirements and professional guidance have recently changed. The counselor wants continuing education that is accurate and relevant.

How should the counselor select the learning resources?

A) Choose the highest-rated commercial course because participant ratings demonstrate legal accuracy

B) Use the counselor's current state's rules because professional standards are uniform across states

C) Verify current requirements with authoritative jurisdictional and professional sources, then select training that addresses those requirements

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Question 27

A school counselor reviews program data and finds that multilingual students with grades comparable to their peers are referred to advanced courses at a much lower rate. Current referrals depend heavily on informal teacher recommendations.

Which response best combines professional development with advocacy for cross-cultural fairness?

A) Encourage multilingual students to request advanced courses without changing the referral process

B) Offer study-skills workshops to multilingual students who were not referred

C) Study culturally responsive identification practices, examine referral criteria with the school team, and advocate for a transparent process

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Question 28

A counselor notices feeling defensive during sessions with clients who communicate assertively. The counselor suspects that this reaction may be influencing which topics are explored but cannot yet identify a consistent pattern.

Which reflective practice would be most useful?

A) Ask assertive clients whether they think the counselor is competent after each difficult exchange

B) Track triggers, internal reactions, interventions, and client responses in a structured reflection and review patterns in supervision

C) Avoid working with assertive clients until the counselor no longer experiences discomfort

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Question 29

A client whose deductible is unmet asks the counselor to submit a code for a longer, more complex session than the 45-minute service actually provided so the plan will pay more. The clinical note accurately reflects the shorter session.

How should the counselor respond?

A) Use the higher-paying code once if the client agrees to repay any later adjustment

B) Add complexity language to the note so the requested code becomes supportable

C) Bill only for the service actually delivered and documented, and explain the financial options honestly

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Question 30

Which treatment-plan entry is an intervention?

A) The client will reduce conflict-related outbursts from four per week to one per week within eight weeks

B) The client will strengthen the ability to manage anger in close relationships

C) The counselor will use role-play to help the client practice assertive requests during weekly sessions

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Question 31

> A client receives counseling by telephone, has limited access to printed materials, and says lengthy written worksheets are difficult to understand. The counselor wants to teach a cognitive coping skill.

How should the counselor adapt the intervention?

A) Email the standard worksheet and spend the next session reading every instruction aloud.

B) Teach one step verbally with a concrete example, practice it together, and use teach-back to check understanding.

C) Postpone skills training until in-person services and complete written materials are available.

Question 32

> Nia receives telehealth counseling for chronic pain and depression. The prior note indicates that she agreed to track sleep, a routine outcome measure increased modestly, and the session revealed no acute safety concern.

What should the counselor do while preparing for Nia's next session?

A) Draft a note describing worsening depression so the session can focus on treatment changes

B) Review the prior goal, sleep-tracking plan, measure change, and questions needed to clarify what changed

C) Omit the outcome measure from preparation because the prior session revealed no crisis

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Question 33

> In an integrated care clinic, a counselor experienced in brief cognitive behavioral interventions meets an older adult with severe dissociation and a complex trauma history. The client requests specialized trauma treatment that the counselor has not been trained to provide.

Which response best addresses counselor-client fit?

A) Explain the limitation, coordinate referral to a qualified trauma clinician, and offer interim support within the counselor's competence

B) Continue brief cognitive behavioral work because a strong alliance can compensate for limited trauma training

C) Give the client a specialist's telephone number and discontinue contact without discussing continuity of care

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Question 34

What is the most appropriate way for a counselor to implement a substantial fee increase for established clients?

A) Give reasonable advance notice, discuss the change and available options, and update the financial agreement

B) Apply the increase immediately so all clients are treated identically on the same billing date

C) Delay notice until claims are processed so clients do not discontinue prematurely

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Question 35

> In a multicultural group, one member values direct disagreement, while another experiences direct challenge as disrespectful and withdraws. Other members begin taking sides about the “right” way to communicate.

What should the group counselor do?

A) Adopt the more direct style because open disagreement produces deeper group work.

B) Name the differing communication meanings and help the group develop norms that allow both engagement and respect.

C) Stop all disagreement until members have developed enough trust to communicate in the same way.

Question 36

> A counselor is beginning secure video services with a client who travels frequently within the counselor's service area. The client assumes that the video platform alone supplies everything needed for emergencies.

Which information should be addressed as part of telemental health intake?

A) Only the client's email address, because location information is unnecessary once identity is verified

B) A guarantee that technical interruptions will never occur during a session

C) How identity and current location will be confirmed, what to do after a connection failure, and how emergency support will be accessed

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Question 37

> A high school student is acutely overwhelmed after witnessing a serious accident. School crisis staff have established that the student is physically safe and has no current intent to harm self or others. The student says, “Everything is happening at once. I can’t think.”

Which intervention is most appropriate at this point?

A) Ask the student to give a complete chronological account so the memory can be processed accurately.

B) Help the student orient to the present, identify one immediate need, and take one manageable step at a time.

C) Explain the diagnostic criteria for acute stress disorder so the student can monitor symptoms.

Question 38

> A college client in counseling for anxiety reports escalating food restriction, dizziness, and rapid weight loss. The counselor recommends prompt evaluation by an eating-disorder specialty team. The client agrees to coordination and signs a release that permits sharing information relevant to the referral.

What should the counselor do next to support continuity of care?

A) Send the specialty team the complete counseling record because a release has been signed

B) Make a timely referral and share the pertinent clinical information authorized by the client

C) Pause all counseling contact until the specialty team completes its evaluation

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Question 39

A wildfire evacuation closes a group practice building, disables the office telephone, and makes paper files temporarily inaccessible. The practice has a tested continuity plan with secure remote contact methods, backup access to essential information, and referral procedures for urgent needs.

What should the clinical director do?

A) Wait until the building reopens so clients do not receive incomplete information

B) Activate the continuity plan, notify clients through approved channels, and prioritize urgent coverage and secure record access

C) Post all scheduled clients' names and appointment changes on the practice's public social-media page

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Question 40

A client identifies a coworker, describes a plan to shoot that person at work the next morning, and reports access to a firearm. The jurisdiction's law requires a counselor who determines that a serious, imminent threat exists to take reasonable protective action, which may include notifying the potential victim and law enforcement.

What is the counselor's most appropriate response?

A) Keep the statement confidential until the counselor obtains the client's written permission to disclose it

B) Follow the jurisdiction's protective-action requirement, continue the safety assessment, and limit disclosures to what is necessary

C) Warn every employee at the workplace so no potentially affected person is omitted

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Question 41

After sending an authorized treatment summary, a counselor discovers that one digit in the fax number was wrong and an unrelated business received it. The practice's incident policy requires immediate reporting to the privacy officer, mitigation when feasible, risk assessment, and any required notifications.

What should the counselor do?

A) Wait to see whether the unintended recipient contacts the practice before creating a record of the event

B) Delete the fax confirmation because retaining it would increase the practice's exposure

C) Report the incident immediately, attempt appropriate mitigation, and follow the required assessment and notification process

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Question 42

> During an intake, a client says, "I am not sure someone from your background will understand what my family expects of me." The counselor feels defensive and is tempted to list prior work with people from the client's community.

What is the most appropriate self-aware response?

A) Reassure the client by describing the counselor's experience with similar cultural groups

B) Redirect to symptoms because discussing cultural differences may weaken the counseling alliance

C) Acknowledge the concern, invite the client to describe what understanding would look like, and discuss preferences and options

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Question 43

> A graduate student seeks help for social anxiety. She values family interdependence and says her most important goal is participating more comfortably at extended-family gatherings, not becoming broadly outgoing. She is willing to try gradual exposure.

How should the counselor apply exposure work most responsively?

A) Use a standard hierarchy focused on speaking to strangers because it offers the broadest practice.

B) Collaboratively build a hierarchy around interactions the client identifies as meaningful at family gatherings.

C) Begin with the largest family event so improvement can occur before avoidance becomes more established.

Question 44

> A client reports repeated disability-related comments from coworkers. The client wants help responding but says, “I’m tired of being told to just think more positively.”

Which intervention best integrates validation and problem solving?

A) Acknowledge the harm, identify what is and is not within the client’s control, and rehearse a response the client chooses.

B) Use cognitive restructuring to show that coworkers’ comments may have been misunderstood.

C) Encourage immediate formal action because self-advocacy requires directly challenging discrimination.

Question 45

A progress note states, "Family reports frequent anger." The counselor later learns that only the client's brother made the statement and that the client disagreed. Which documentation action best supports accurate continuity of care?

A) Add a dated clarification attributing the statement to the brother and noting the client's disagreement

B) Replace the original sentence with "The client has anger problems" so future clinicians recognize the issue

C) Delete the statement entirely because the available accounts conflict

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Question 46

> During a telehealth crisis session, a distressed client becomes more agitated each time the video freezes and says, “I can’t keep repeating myself.” The counselor had confirmed the client’s location and backup telephone number at the start of the session.

What should the counselor do next?

A) Continue by video and ask the client to summarize the story in shorter statements.

B) End the session and send coping resources electronically to avoid further frustration.

C) Acknowledge the added frustration, offer to switch to the agreed telephone backup, and reestablish a manageable pace.

Question 47

> A client says punctuality is essential to being dependable but has arrived more than 20 minutes late for the last three sessions. The client has not mentioned the pattern.

Which counselor response is the most effective use of confrontation?

A) “You are not as dependable as you believe, because you repeatedly arrive late.”

B) “You value punctuality, and I’ve noticed you have arrived late recently. How do you understand that difference?”

C) “I will overlook the late arrivals because confronting them could weaken our relationship.”

Question 48

> A 15-year-old is stepping down from an intensive outpatient program after improved mood and school attendance. Continued success depends on medication follow-up, school accommodations, family support, and weekly outpatient counseling. The adolescent and guardian consent to coordination.

Which transition approach best supports continuity?

A) Give the family a list of providers and let each service develop unrelated goals

B) Arrange a warm handoff with shared transition goals, confirmed appointments, and defined provider roles

C) Delay discharge until the adolescent can function without school or family supports

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Question 49

Which combination provides the most useful basis for evaluating treatment effectiveness at planned intervals?

A) Repeated symptom or behavior data, functioning indicators, and the client's view of progress

B) The counselor's impressions and the number of sessions the client has attended

C) The original diagnosis and whether the client completes every assigned activity

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Question 50

> During a school counseling session, 15-year-old Sam folded their arms and was silent for about two minutes after discussing a schedule change. Sam then said, "I'm angry because nobody listened to me."

Which note entry is most accurate and neutral?

A) Sam was oppositional and refused to cooperate because of the schedule change.

B) Sam displayed conduct problems that require disciplinary intervention.

C) Sam folded their arms and was silent for about two minutes, then reported feeling angry and ignored after the schedule change.

Question 51

> An approved AI drafting tool generates a treatment-plan paragraph that includes a diagnosis the counselor never made and a medication the client does not take. The organization requires clinicians to verify all generated content before it enters the record.

What should the counselor do?

A) Save the paragraph unchanged because use of an approved tool transfers responsibility to the organization

B) Ask the client to approve the paragraph so the inaccuracies become part of shared decision-making

C) Remove unsupported content, verify the remaining information, and accept personal responsibility for the final entry

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Question 52

> During an urgent assessment, a client states an intention to seriously harm a named former partner tonight, describes waiting near the person's workplace, and reports having a firearm in the car. The client refuses to agree to any immediate safety steps.

What should the counselor do next?

A) Initiate emergency safety procedures and a higher level of intervention while following applicable protective duties

B) Schedule a routine appointment for the next week because the threat has not yet been acted upon

C) Focus on a projective assessment to determine whether the statement symbolically represents anger

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Question 53

> A Black client tells a White counselor, “When I mentioned racism at work, you focused on my communication style. I don’t think you understand what I’m dealing with.”

Which response is most likely to begin repairing the alliance?

A) “I treat every workplace conflict the same way, regardless of anyone’s racial background.”

B) “I hear that my response missed the role of racism and left you feeling misunderstood. Would you tell me more about the impact?”

C) “I may not fully understand, so it would be better for us to avoid interpreting the event until I learn more about your culture.”

Question 54

> A client with depression set an objective of walking outdoors for 20 minutes on five days each week. After three weeks, the client reports that knee pain limits walking but says seated exercise is manageable and still supports the broader goal of increasing activity.

What is the best treatment-planning response?

A) Keep the walking objective because changing it would make the original outcome data unusable

B) Revise the activity objective collaboratively to remain measurable and attainable while addressing the same broader goal

C) Replace the activity goal with a pain-focused diagnosis before making any further treatment changes

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Question 55

> A college athlete wants to stop sending hostile messages during conflicts. The client recognizes tension in the jaw and hands just before acting and likes practicing skills through drills rather than discussion alone.

Which in-session intervention is the best fit?

A) Rehearse noticing the physical cues, taking a timed pause, and composing a response only after arousal decreases.

B) Explore childhood experiences associated with anger without practicing a behavioral alternative yet.

C) Advise the client to suppress angry thoughts whenever physical tension begins.

Question 56

A vendor offers an AI note assistant that will retain identifiable session audio to train its general model. The group practice's policy prohibits uploading identifiable client information to an outside tool until privacy, security, contracting, and clinical reviews are complete.

What is the counselor's best course of action?

A) Use the tool after verbally telling clients that artificial intelligence may improve note quality

B) Decline to upload client data unless the tool completes the required review and receives practice approval

C) Use the tool for established clients because their general consent for treatment covers documentation methods

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Question 57

A client has sustained improvement in depression and is preparing to discharge from outpatient counseling. The client recalls that withdrawal from friends and missing morning classes preceded the last worsening episode.

Which discharge-plan element would most directly support maintenance of gains?

A) A promise to return to counseling only if symptoms become as severe as before

B) A written plan linking early warning signs to coping steps, support contacts, and follow-up options

C) A summary listing every intervention the counselor used during the course of treatment

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Question 58

A counselor discovers that the credential specifically required by the state board to supervise prelicensed counselors expired last week. The board prohibits supervision under an expired credential, and two supervisees have sessions scheduled today.

What is the counselor's most appropriate action?

A) Continue temporarily because the counselor's underlying clinical license remains active

B) Stop providing supervision under the expired credential and arrange qualified coverage while seeking reinstatement

C) Ask the supervisees to document the meetings as peer consultation until the credential is renewed

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Question 59

A payer requests records supporting three claims during an audit. The provider agreement permits such audits, and the practice verifies the requester's identity and authority. The request is limited to documentation supporting the billed services.

How should the counselor respond?

A) Transmit the requested supporting documentation through an approved secure method and record the disclosure

B) Refuse all disclosure because payment audits never justify access to clinical documentation

C) Send the client's complete lifetime record so the payer cannot claim that relevant information was withheld

Question 60

Which set of resources generally provides the strongest foundation for learning about an emerging counseling intervention?

A) Current practice guidelines, peer-reviewed research, and training led by qualified professionals

B) Popular social media discussions, client testimonials, and a widely followed influencer's course

C) Vendor-sponsored case examples, promotional outcome summaries, and informal peer enthusiasm

Question 61

A private practitioner wants more online reviews and considers asking current clients during sessions to post testimonials. The counselor recognizes that clients may feel pressured because of the power difference and that public reviews can expose the reviewers' client status.

Which marketing approach is most appropriate?

A) Ask only clients who have already reported substantial improvement to post reviews

B) Avoid soliciting testimonials from current clients and use accurate, non-client-specific information in advertising

C) Offer a small fee credit to any client who posts a review without naming the counselor's specialty

Question 62

A practice hires an outside billing company. The company asks for unrestricted access to all psychotherapy content, although it needs only limited clinical and demographic information for claims. Practice policy requires a compliant service agreement and role-based access for vendors.

What should the counselor advocate for?

A) A compliant agreement and access limited to the information the billing company needs to perform its authorized work

B) Full access because any company involved in payment automatically becomes part of the treatment team

C) No access to client information because billing functions can never be performed by an outside service

Question 63

An approved digital screening tool labels a client's suicide risk as low, but the client's written responses describe a current plan and access to lethal means. The vendor warns that the risk label is only decision support and must be clinically reviewed.

What is the counselor's best response?

A) Accept the low-risk label because consistent use of the approved tool reduces clinician bias

B) Conduct a direct risk assessment, act on the clinical evidence, and document any decision that differs from the tool

C) Repeat the same digital screening until the software produces a result consistent with the written responses

Question 64

A Deaf client who primarily communicates in American Sign Language schedules an intake at a community counseling clinic. The client's spouse offers to interpret, although the client has not requested the spouse's help.

What should the counselor arrange before the intake?

A) A qualified behavioral health ASL interpreter, while asking the client about the preferred communication setup

B) Interpretation by the spouse because the spouse already knows the client's history

C) Written communication only because it avoids involving another person in the session

Question 65

An adult client requests a copy of the clinical record. The applicable rule gives the client access within 15 days unless the practice documents a permitted basis for denial and provides the required review process. The counselor is uncomfortable because several entries contain informal shorthand, but identifies no risk of harm or other lawful basis to deny access.

What should the counselor do?

A) Deny access because informal wording could cause the client to misunderstand the treatment

B) Rewrite the entries into polished summaries and provide only the rewritten version

C) Provide access within the required period while preserving the original record and following the practice's access procedure

Question 66

A client experiencing intense anxiety agrees to try paced breathing. After two cycles, the client says, “This is making me dizzy,” and appears more frightened. The client remains able to communicate and is physically safe.

How should the counselor respond?

A) Encourage several more cycles because initial discomfort is expected when learning the skill.

B) Pause the breathing exercise, shift to an external grounding method, and monitor the client’s response.

C) Explain that dizziness is caused by anxiety and ask the client to disregard it while continuing.

Question 67

After several months with an unusually high caseload, a counselor is sleeping poorly, becoming irritable with coworkers, and discovering omissions in recent progress notes. No client emergency is currently known.

What is the best response?

A) Promptly address workload and care quality with a supervisor, correct affected records, and establish sustainable supports and recovery practices

B) Schedule a vacation several months away and maintain the current workload until then

C) Work longer hours to eliminate the backlog before telling anyone about the strain

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Question 68

Two members of an outpatient group discover that they work in the same large organization. Neither supervises the other, but both worry about encountering each other at work.

What is the group leader’s best response?

A) Require one member to transfer to another group to eliminate any possibility of contact.

B) Ask both members to tell the group which department they work in so risks can be evaluated openly.

C) Review confidentiality limits and help the members develop a mutually acceptable plan for incidental contact.

Question 69

Twelve days after surviving a serious motor vehicle crash, a client reports intrusive memories, distressing dreams, flashbacks, negative mood, detachment, inability to recall part of the crash, avoidance of thoughts and driving, marked startle, and impaired work functioning. The symptoms began the day of the crash, and the client reports no previous trauma-related disorder.

Which diagnosis is most consistent with the duration and symptom pattern described?

A) Posttraumatic stress disorder

B) Acute stress disorder

C) Adjustment disorder with anxiety

Question 70

Before using a translated screening instrument with a client population, which evidence is most important to review?

A) Whether the translated version has the same number of words as the original

B) Whether reliability, validity, and norms are adequate for the intended language group and purpose

C) Whether the instrument is popular among counselors in unrelated settings

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Question 71

> A client says a mindfulness exercise feels inconsistent with the client's religious beliefs and worries that continuing it would be disrespectful to their faith.

What is the counselor's most culturally responsive next step?

A) Explain that mindfulness is a clinical exercise rather than a religious practice and continue using it.

B) Discontinue all attention-based exercises because the client's concern makes them inappropriate.

C) Explore the client's concern and collaborate on an acceptable practice, such as faith-consistent reflection or sensory grounding.

Question 72

> A Deaf adult requests an American Sign Language interpreter for an intake appointment. The client's hearing sibling offers to interpret and says this would be easier for the family.

What is the counselor's best response?

A) Arrange a qualified interpreter and discuss communication preferences directly with the client

B) Accept the sibling as interpreter because relatives are more familiar with the client's history

C) Conduct the intake through written notes without asking whether that method is effective for the client

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Question 73

> During family counseling, two parents argue and each asks their 14-year-old, "Tell the counselor who is right." The adolescent looks down and says, "I don't want to choose."

What should the counselor do?

A) Ask the adolescent to describe each parent's strongest argument so the family can feel equally represented.

B) Remove the adolescent from the referee role and facilitate direct communication between the parents.

C) Invite the adolescent to leave the room while the counselor determines which parent's position is more reasonable.

Question 74

> A 72-year-old client presents with new confusion, slowed responses, and difficulty recalling the date. The client's adult child reports that these changes began two days after a primary care clinician adjusted several medications. The client was functioning independently the previous week.

What is the best assessment response by the counselor?

A) Diagnose a neurocognitive disorder based on the observed disorientation

B) Arrange prompt medical evaluation while assessing immediate safety and documenting the acute change

C) Complete a personality inventory to determine whether the confusion reflects psychological resistance

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Question 75

> Two separated parents and their 13-year-old arrive for family counseling. Each parent assumes the counselor will keep a separate confidential file and advocate for that parent's preferred custody arrangement.

What should the counselor clarify before beginning services?

A) The parent who scheduled the appointment is automatically the only client

B) Who the client or clients are, the counselor's role, how records and information will be handled, and the limits of confidentiality

C) Family counseling requires the counselor to determine which parent should receive custody

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Question 76

> A 19-year-old client leaving foster care has authorized a wraparound meeting with a housing navigator and an employment coach. The client wants to keep a new job but has repeatedly missed shifts because evening bus service ends too early.

Which approach would make the meeting most useful?

A) Allow the providers to exchange all available information so they can design a comprehensive plan

B) Center the client's employment goal, share the relevant transportation barrier, and define each participant's follow-up role

C) Limit the meeting to a summary of symptoms because nonclinical providers should not discuss practical barriers

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Question 77

A client's score is at the 84th percentile on an appropriately normed measure. What does this result mean?

A) The client answered 84% of the items correctly

B) The client's score is 84 standard deviations above the mean

C) The client's score equals or exceeds the scores of about 84% of the norm group

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Question 78

Which intervention best fits a client who recognizes consequences of substance use but remains uncertain about making a change?

A) Explore the client's own reasons for and against change while supporting autonomy.

B) Develop a detailed abstinence schedule and ask the client to commit to it immediately.

C) Challenge each statement favoring continued use until the client accepts the need to change.

Question 79

> A Deaf client uses a qualified American Sign Language interpreter during counseling. The counselor realizes that most questions and eye contact have been directed toward the interpreter rather than the client.

What adjustment is most appropriate?

A) Address and look toward the client while allowing the interpreter to facilitate communication.

B) Continue addressing the interpreter because the interpreter is responsible for conveying the meaning accurately.

C) Alternate questions between the client and interpreter so both have equal roles in the counseling dialogue.

Question 80

> A client's grief resembles a counselor's unresolved experience after a parent's death. The counselor notices an urge to protect the client from painful topics and has repeatedly redirected the conversation when the client begins discussing anger toward the deceased parent.

How should the counselor respond to this pattern?

A) Examine the possible countertransference in supervision and monitor how it is shaping interventions

B) Describe the counselor's own loss in detail so the client understands the source of the reactions

C) End counseling immediately because any personal similarity prevents effective work

---

Question 81

> A long-term client invites the counselor to attend the client's graduation, saying, "You're the reason I made it." The counselor's role and agency boundaries make attendance inappropriate.

What is the most therapeutic response?

A) Acknowledge the significance of the invitation, explore what attendance would represent, and discuss an appropriate way to honor the milestone in counseling.

B) Decline briefly without further discussion so the professional boundary is not made emotionally confusing.

C) Accept the invitation because attending a public ceremony does not create a meaningful boundary concern.

Question 82

A 10-year-old is referred for possible attention-deficit/hyperactivity disorder. The caregiver reports chronic snoring and insufficient sleep, and the teacher notes that the child often seems not to hear instructions. No hearing or sleep evaluation has occurred.

Which assessment plan is best?

A) Gather symptom and impairment data across settings while evaluating developmental, hearing, sleep, and other possible contributors

B) Diagnose attention-deficit/hyperactivity disorder from the teacher's report because school impairment is sufficient by itself

C) Exclude attention-deficit/hyperactivity disorder entirely because snoring always accounts for inattentive behavior

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Question 83

Which plan most effectively develops competence in an empirically supported counseling approach?

A) Read the treatment manual and use the approach whenever its title matches a client's diagnosis

B) Complete skills-based training, obtain supervised feedback, and monitor client response while consulting current evidence

C) Observe several online demonstrations and rely on clinical intuition to modify the approach

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Question 84

Marisol, a university counseling center client, experiences panic symptoms while commuting and in crowded lecture halls. She has authorized the counselor in writing to coordinate with a disability access consultant and her academic advisor. Her stated goal is to attend required laboratory sessions consistently.

Which information should the counselor prepare to communicate at the coordination meeting?

A) Marisol's full diagnostic intake so the stakeholders can independently determine accommodations

B) A summary of functional barriers, current coping supports, and Marisol's stated attendance goal within the authorization

C) A request that the stakeholders recommend a new counseling goal before hearing Marisol's preferences

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Question 85

During a telehealth session, a counselor notices that the platform's automatic cloud-recording feature is on. The client consented to live video counseling but not to recording. Practice policy permits recordings only with specific written consent and approved encrypted storage.

What should the counselor do?

A) Continue recording because consent to telehealth includes consent to the platform's default features

B) Finish the session and ask the client afterward whether the recording may be retained

C) Stop the recording, address the unauthorized file under policy, and obtain specific consent before any future recording

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Question 86

A client with no history of mood episodes reports four days of very little sleep, unusually rapid speech, and increased activity. The symptoms began soon after a medical provider started a corticosteroid. The client denies prior similar periods.

What is the best next step in the diagnostic assessment?

A) Evaluate the temporal relationship to the medication and coordinate medical review before assigning a primary bipolar diagnosis

B) Diagnose bipolar I disorder because reduced sleep and rapid speech are sufficient to establish a manic episode

C) Diagnose insomnia disorder because sleep loss is the only symptom that requires assessment

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Question 87

For eight months, a client has experienced excessive, difficult-to-control worry about finances, family health, work performance, and routine errands. The client also reports restlessness, muscle tension, irritability, and disrupted sleep. The symptoms cause substantial distress and are not attributable to a substance or medical condition.

Which diagnosis best fits the information provided?

A) Generalized anxiety disorder

B) Adjustment disorder with anxiety

C) Panic disorder

Question 88

A client completing outpatient treatment for alcohol use disorder identifies payday gatherings with former drinking partners as a high-risk situation. The client has a supportive cousin and has practiced leaving risky settings early.

Which relapse-prevention plan is most complete?

A) Identify warning signs, plan alternatives for payday, rehearse contacting the cousin, and specify what to do after a lapse

B) Advise the client to avoid all social events indefinitely and regard any lapse as treatment failure

C) Ask the client to rely on motivation and return only if regular alcohol use resumes

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Question 89

When is a closed question generally most useful during a counseling interview?

A) When clarifying a specific fact after the client has had room to describe the experience.

B) When encouraging the client to discover an unrecognized personal meaning.

C) When demonstrating empathy for an emotionally complex experience.

Question 90

Which information is most important to include in telehealth informed consent before routine services begin?

A) A promise that encryption will prevent every possible privacy or technology problem

B) Privacy and technology risks, service limits, backup communication steps, and emergency procedures

C) A statement that telehealth follows the counselor's home-state rules regardless of the client's location

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Question 91

A university student attends an intake after a conduct office recommended counseling. The student says, “I was told I have no choice, and you will report whether I admit I caused the incident.” The referral document actually requests only attendance confirmation, and counseling participation is voluntary.

What should the counselor do?

A) Clarify the voluntary nature of counseling, the limited attendance disclosure, and the student's options before seeking consent

B) Begin questioning the student about the incident before correcting the misunderstanding

C) Tell the student that a conduct referral eliminates confidentiality for the entire counseling record

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Question 92

An 8-year-old's teacher reports frequent inattention and incomplete work, while a caregiver reports focused behavior at home. A brief rating scale completed by each adult produces similarly discrepant results.

How should the counselor interpret the information?

A) Examine differences across settings and informants, obtain developmental and school data, and avoid drawing a conclusion from one reporter

B) Accept the teacher's rating as definitive because academic settings always provide the most valid behavioral observations

C) Average the two rating scores and diagnose solely from the resulting number

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Question 93

An employee contacts a counselor after a supervisor recommended the agency's employee assistance program because of repeated lateness. The employee believes the counselor will send the supervisor a complete account of everything discussed.

What should the counselor clarify during intake?

A) The supervisor automatically becomes the counseling client because the supervisor initiated the referral

B) What information, if any, may be shared, the limits of confidentiality, and the employee's choices about participation

C) All session details must be released to the employer whenever work performance prompted the referral

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Question 94

In a counseling group, Malik describes feeling ashamed after losing his job. Another member quickly lists job-search strategies. Malik becomes quiet, looks down, and pulls his chair back.

What is the group counselor’s best intervention?

A) Ask the advice-giving member to finish the list so Malik can decide which suggestions are useful.

B) Shift the discussion to another member who has also experienced unemployment.

C) Pause the advice and invite Malik to describe what happened for him as the group responded.

Question 95

Which statement best distinguishes vicarious trauma from occupational burnout?

A) Vicarious trauma occurs only after direct exposure to danger, whereas burnout occurs after hearing clients' accounts

B) Vicarious trauma can alter beliefs and worldview through empathic exposure, whereas burnout more often reflects chronic occupational strain and exhaustion

C) Vicarious trauma is a temporary loss of motivation, whereas burnout is a diagnosable trauma disorder

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Question 96

> While describing the death of a close friend, a client stops speaking and begins to cry. After several seconds, the client remains oriented, takes a slow breath, and looks toward the counselor.

What is the counselor's best immediate response?

A) Remain quietly attentive and allow the client space to continue when ready.

B) Ask a series of factual questions about the friend to help the client regain composure.

C) Reassure the client that grief becomes easier with time and move to coping strategies.

Question 97

> A counselor notices two days later that a progress note omitted a clinically important medication change discussed in session. The electronic record locks signed notes but permits dated addenda.

How should the counselor correct the record?

A) Enter a current, clearly labeled addendum that supplies the omitted information without changing the original date

B) Ask the system administrator to unlock the note and insert the information as if it were documented on the session date

C) Leave the note unchanged because any late entry would make the record less reliable

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Question 98

> A client has made little progress on a goal requiring three evening activities each week. The client recently became the primary caregiver for an older relative and says the original schedule is no longer realistic, although the overall goal remains important.

Which response best reflects appropriate plan revision?

A) Collaboratively adjust the objective and supports to fit the new caregiving demands while preserving the valued goal

B) Retain the objective for another month so that progress can be compared with the original baseline

C) Remove the goal because a change in circumstances indicates that the client is not ready for treatment

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Question 99

An adult client asks a counselor to delete an accurately documented diagnosis because the client is worried about stigma. The clinic's amendment policy requires a clinical review, a written reason for any denial, and an opportunity for the client to add a statement of disagreement to the record.

How should the counselor address the request?

A) Review the request, explain a supported denial in writing, and offer to add the client's statement of disagreement

B) Delete the diagnosis because concern about stigma requires the counselor to grant any requested amendment

C) Refuse to discuss the request because clients cannot challenge information entered by a clinician

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Question 100

A client repeatedly describes the counselor as the only person who truly understands and asks for extra contact between sessions. The counselor enjoys feeling indispensable and has begun allowing this client's sessions to run longer than other clients' sessions.

What should the counselor do?

A) Recognize the reinforcement of the boundary drift, seek supervision, and restore a clinically appropriate structure

B) Continue the longer sessions because the client's positive transference strengthens the alliance

C) Tell the client that the praise is manipulative and prohibit any discussion of the relationship

---

Question 101

Assessment findings support social anxiety disorder for a client whose main goal is to participate in staff meetings. The client currently avoids speaking and rates anticipated distress as high but manageable with support.

Which initial plan best integrates the assessment findings with the client's goal?

A) Develop a graded sequence of meeting-related speaking tasks and track distress and participation

B) Focus on general self-esteem until the client reports feeling fully confident in all social settings

C) Recommend an unstructured social group without connecting attendance to the workplace goal

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Question 102

> Aisha, a community agency client facing eviction, has authorized coordination with a school family liaison, a housing advocate, and a food-support program. At a meeting, the team discovers that two members started duplicate benefit applications and no one is helping Aisha assemble the documents requested by the landlord.

What should the counselor facilitate next?

A) Confirm Aisha's priorities, assign nonduplicative responsibilities and deadlines, and document the follow-up plan

B) Ask every team member to handle all applications independently to maximize the chance of success

C) Have the school liaison choose the plan because that professional has the most frequent contact with the family

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Question 103

> A client who uses a wheelchair prefers in-person counseling, but the agency's counseling rooms are reached only by stairs. A manager suggests that the counselor simply offer telehealth. The counselor realizes that professional knowledge about disability-responsive practice is limited.

Which action best addresses both professional growth and client advocacy?

A) Explain that telehealth is clinically equivalent and therefore resolves the access concern

B) Advocate for an accessible service option while seeking training and consultation on disability-responsive practice

C) Refer the client to another agency solely because the counselor lacks experience with wheelchair users

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Question 104

> A counselor is proficient with depression and anxiety screeners but is asked to add a complex personality inventory to routine intake. The counselor has no training in selecting, administering, scoring, or interpreting that instrument.

How should the counselor respond?

A) Identify the competence gap and obtain appropriate training and consultation or refer that assessment component before using the instrument

B) Administer the inventory because standardized instructions eliminate the need for specialized competence

C) Use the inventory only with clients whose presentations appear straightforward and seek help for unusual profiles

---

Question 105

A counselor leaving a solo practice learns that the professional liability policy is claims-made and may not cover a claim first reported after the policy ends unless extended reporting coverage applies. No claim is currently known.

What should the counselor do before ending the policy?

A) Cancel immediately because liability ends when the practice stops accepting clients

B) Rely on the next employer's policy to cover all work performed in the former practice

C) Confirm appropriate tail or prior-acts arrangements with the insurer and retain the coverage documentation

Question 106

A counselor in independent practice is creating a professional will in case sudden incapacity prevents communication with clients.

Which provision most directly promotes continuity of care?

A) Designating a qualified professional to secure records and notify clients about care options

B) Authorizing a family member to review records and decide which clients need further treatment

C) Directing that all clinical records be destroyed immediately if the counselor becomes incapacitated

Question 107

Which mental status examination component is best illustrated by asking a client what the client would do after finding a stamped, addressed envelope on the sidewalk?

A) Remote memory

B) Fund of knowledge

C) Judgment

Question 108

A young adult with a recently acquired mobility impairment feels anxious about requesting an accessible workspace. The client wants to speak for themself in an upcoming meeting but does not know how to begin.

Which counseling intervention best promotes client empowerment?

A) Help the client choose the request, role-play the conversation, and refine wording based on the client’s feedback.

B) Contact the employer for the client because professional advocacy is more effective than self-advocacy.

C) Provide a script for the client to memorize exactly so anxiety does not interfere with the meeting.

Question 109

> Before a college counseling session, a counselor reviews a newly completed outcome measure showing lower anxiety. The prior note says the client expected anxiety to worsen before an upcoming class presentation.

How should the counselor use the apparently different information?

A) Assume the client has improved and cancel further review of presentation-related anxiety

B) Bring both data points into the session, ask about their context, and use the clarification to guide and document the current assessment

C) Revise the prior note so it agrees with the more recent outcome-measure score

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Question 110

> After a neighborhood fire, a parent asks how to explain the event to an 8-year-old who repeatedly asks whether another fire will happen. The child is safe and is showing no severe impairment.

Which guidance is most appropriate?

A) Avoid discussing the fire unless the child raises it, because explanations can reinforce fear.

B) Give the child detailed information about fire probability so the child can evaluate the actual risk.

C) Use simple truthful language, describe current safety routines, practice a coping skill, and check what the child understood.

Question 111

> After medical causes have been ruled out, a college student who recently experienced a first panic episode says, “My racing heart means I’m losing control.” The student is calm enough to talk and asks what happened.

Which counselor response best combines psychoeducation with skill development?

A) Explain the body’s alarm response in plain language, then rehearse a brief grounding strategy and ask the student to describe its effect.

B) Provide a handout on panic symptoms and ask the student to read it before the next counseling appointment.

C) Emphasize that the episode was harmless and advise the student to ignore similar physical sensations in the future.

Question 112

> Four days after a psychiatric nurse practitioner increased Leila's medication, Leila tells her counselor that she feels light-headed and has trouble concentrating. She reports no falls or other emergency symptoms, and the counselor observes a steady gait. A current written authorization permits coordination with the prescriber.

How should the counselor handle this information?

A) Recommend that Leila return to the previous dose until her next appointment

B) Record "medication intolerance" as the counselor's conclusion and continue routine sessions

C) Document the timing and effects as Leila reported them and contact the prescriber to relay the concern

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Question 113

> During a couple session, each partner gives a conflicting account of a recent argument and repeatedly interrupts to correct the other. Both insist that the counselor decide who remembers the event accurately.

Which intervention best serves the counseling process?

A) Compare the details of both accounts and identify which partner provides the more consistent version.

B) Structure equal speaking turns and reflect the pursue-withdraw pattern occurring between the partners.

C) Ask each partner to submit a written account after the session so the counselor can determine what happened.

Question 114

> A counselor receives a subpoena from an attorney requesting a former client's complete record. The client has not authorized disclosure. In this jurisdiction, a subpoena issued by an attorney does not by itself override counselor-client privilege, and the agency's policy directs staff to seek legal review rather than ignore legal process.

What is the counselor's most appropriate response?

A) Send only the progress notes because they are less sensitive than the full record

B) Preserve the record, notify the client when permitted, and obtain legal guidance about asserting privilege or responding

C) Refuse the request in writing and destroy duplicate copies so they cannot be subpoenaed later

---

Question 115

When arranging translation of a written counseling resource into a client's preferred language, which approach best supports accurate access?

A) Ask a bilingual staff member in an unrelated role to translate informally and assume the client's literacy level

B) Use a qualified translator, preserve the meaning in plain language, and provide a way to ask questions through qualified language support

C) Rely on machine translation without review so the resource can be delivered as quickly as possible

---

Question 116

A couple requests conjoint counseling after one partner disclosed ongoing intimidation, monitoring of communications, and fear of retaliation for speaking openly. The fearful partner says it would not be safe to discuss the relationship honestly in joint sessions.

How should the counselor use this information in planning services?

A) Begin conjoint counseling with a rule that neither partner may discuss sessions afterward

B) Use communication-skills training in the first joint session to reduce the power imbalance

C) Avoid conjoint work at this time and develop a safety-focused referral or individual service plan

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Question 117

Priya, an uninsured community mental health client, is seeking outpatient eating-disorder care and has transportation only on Saturdays. A directory entry from nine months ago lists a program as offering sliding-scale weekend appointments, but the counselor has not recently confirmed the information. Priya reports no acute medical or safety concern.

What should the counselor do before recommending the program?

A) Send Priya to the program based on the existing directory entry so the referral is not delayed

B) Refer Priya directly to inpatient care because access barriers make outpatient treatment unsuitable

C) Verify current eligibility, fees, scheduling, and accessibility without disclosing Priya's identity, then review the options and next steps with her

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Question 118

In an outpatient therapy group, one member reports that another member described a third member's personal story at a community event. At orientation, the counselor explained that members are expected to protect confidentiality but that the counselor cannot guarantee every member's conduct.

How should the counselor respond within the group?

A) Guarantee that no further disclosure will occur because the rule has now been reinforced

B) Address the reported breach, assess its impact, and revisit confidentiality expectations and limits with the group

C) Disband the group immediately because any member disclosure makes continued group counseling unethical

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Question 119

A client with insomnia works rotating shifts and lives in a multigenerational household where evening time together is highly valued. A standard fixed bedtime recommendation has not been feasible.

What is the counselor's best intervention adjustment?

A) Emphasize that evidence-based sleep routines require the client to prioritize a fixed bedtime over family activities.

B) Collaborate on consistent sleep cues and stimulus-control practices that can flex with shifts and preserve valued family time.

C) Stop using behavioral sleep strategies because the client's schedule prevents adequate treatment adherence.

Question 120

During a random renewal audit, a counselor realizes that two continuing education certificates cannot be located. The counselor attended the programs but cannot independently verify completion before the board's deadline.

What should the counselor do?

A) Report the documentation accurately and request the board's instructions for resolving the deficiency

B) Recreate the certificates from calendar entries and submit them as replacement documentation

C) Omit the two programs and certify that all renewal requirements have nevertheless been documented

Question 121

A 14-year-old client reports that a caregiver struck the client repeatedly with a belt two days ago, leaving visible bruises. The state law provided to counselors requires a report when there is reasonable suspicion of child abuse; proof is not required.

What should the counselor do?

A) Make the required report promptly and explain the confidentiality limit to the client in a supportive manner

B) Wait until the counselor interviews the caregiver and determines whether the account is credible

C) Maintain confidentiality because reporting without proof could damage the counseling relationship

Question 122

What is the primary therapeutic purpose of a counselor's summary near the end of a session?

A) To introduce new interpretations that the client can consider between sessions.

B) To document every topic discussed so no session content is omitted.

C) To integrate major themes, check shared understanding, and support continuity.

Question 123

> Omar lives in a rural area and seeks trauma-focused counseling with evening availability. He is uninsured, has limited transportation, and cannot rely on the public library's internet connection. The counselor's recently verified directory includes a mobile clinic and a low-bandwidth telehealth program.

What is the best next step in making a referral?

A) Review the eligibility and access tradeoffs with Omar, then offer a warm connection to his preferred verified service

B) Select the program with the strongest clinical reputation regardless of its hours, cost, or access requirements

C) Give Omar the complete directory and ask him to call programs until one accepts him

---

Question 124

> An autistic adult seeks help managing workplace overload. The client explains that fluorescent lighting and unpredictable verbal instructions contribute to shutdowns and prefers written agendas and concrete planning. The client wants to remain in the current job.

Which treatment-planning response is most appropriate?

A) Prioritize increasing tolerance of sensory discomfort before discussing workplace changes

B) Build concrete regulation and self-advocacy objectives around the client's sensory needs and communication preferences

C) Recommend leaving the job because environmental triggers cannot be addressed through counseling

---

Question 125

> In a video session, a client says, “Everything is fine,” while looking away, speaking more softly, and becoming unusually still. The counselor cannot tell whether the behavior reflects emotion, distraction, or a poor connection.

What is the counselor’s best response?

A) Interpret the behavior as avoidance and ask why the client is withholding emotion.

B) Accept the verbal statement because nonverbal behavior is unreliable in telehealth.

C) Describe the observable change without assigning meaning and invite the client to clarify what is happening.

Question 126

A client managing chronic pain says that spiritual practices are an important source of endurance but has not said whether these practices should be part of counseling. The counselor wants the plan to reflect the client's worldview without making assumptions.

What is the best planning step?

A) Add participation in a faith community as an objective because spirituality is a stated strength

B) Ask how, if at all, the client wants spiritual resources incorporated into goals and supports

C) Keep spirituality out of the plan because it is outside the scope of professional counseling

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Question 127

At intake, a client describes insomnia, conflict with a sibling, overdue bills, and worry about job performance. The client says, “If I could sleep, I could think clearly enough to handle the rest,” and reports feeling ready to change the bedtime routine.

Which planning response best supports collaborative prioritization?

A) Begin with the overdue bills because financial stress is the most concrete concern

B) Agree to target sleep first while noting how the other concerns will be monitored

C) Address the sibling conflict first because interpersonal stress may be causing the insomnia

---

Question 128

Eli brings two bottles of the same antidepressant in different strengths to a counseling appointment and says he alternates them "depending on the day." He reports no acute symptoms. The counselor cannot verify the current prescription, but a written authorization permits contact with the prescriber.

What should the counselor do?

A) Choose the newer bottle and instruct Eli to discard the older medication

B) Advise Eli to continue his current pattern until the next scheduled prescriber visit

C) Record Eli's exact report and the bottle labels, encourage prompt clarification from the prescriber or pharmacist, and coordinate under the authorization

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Question 129

> At a college counseling intake, a student reports recent superficial cutting and says, “Sometimes I wish I could disappear,” but adds that the cutting is not intended to cause death. The student appears calm and is willing to continue talking.

What is the counselor's most appropriate assessment response?

A) Assume there is no suicide risk because the student distinguishes cutting from a suicide attempt

B) Ask directly about suicidal thoughts, intent, plan, access to means, past behavior, and protective factors

C) End the interview and arrange involuntary hospitalization solely because any self-injury indicates imminent suicide risk

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Question 130

A counselor wants to know whether a measure produces similar scores when the same stable construct is assessed on two occasions. Which psychometric property is most directly relevant?

A) Content validity

B) Interrater agreement

C) Test-retest reliability

Question 131

> A client who uses a power wheelchair contacts a community counseling center. The office entrance is accessible, but the client reports that the standard intake room is difficult to navigate and asks whether the first meeting can occur by secure video instead.

Which intake response is most appropriate?

A) Ask about the client's access preferences, explain the available service formats, and confirm that the selected format can meet the client's needs

B) Schedule the standard intake room because all clients should complete the same intake process before accommodations are considered

C) Refer the client elsewhere immediately because a request for video intake indicates that in-person services are unsuitable

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Question 132

> A counselor has provided therapy to one parent and a child for six months. An attorney asks the counselor to conduct a custody evaluation and recommend which parent should receive primary custody. The counselor has no forensic evaluation role, and the request would require evaluating people who are not counseling clients.

What should the counselor do?

A) Decline the custody evaluator role, clarify the limits of the treatment role, and provide only properly authorized factual information

B) Accept the role because the existing therapeutic relationship provides the most extensive knowledge of the family

C) Recommend the counseling client for custody while noting that the other parent was not evaluated

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Question 133

Which approach is most appropriate when a counseling practice establishes a record-retention and destruction schedule?

A) Destroy every record when treatment ends so no information remains vulnerable to disclosure

B) Keep every record permanently because indefinite retention creates the strongest legal protection

C) Apply the governing laws and contracts, document the schedule, and use secure destruction after the required period

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Question 134

When responding to a client's valid authorization to release counseling information, which practice best protects confidentiality?

A) Release the entire record so the recipient can decide which portions are relevant

B) Verify the authorization and disclose only the information covered and reasonably needed for its stated purpose

C) Summarize the record from memory so no original documentation leaves the practice

---

Question 135

> At a scheduled six-week review, a client's standardized anxiety scores and work attendance are unchanged. The client has attended consistently but says the current plan focuses on relaxation while the main concern is avoiding difficult conversations with a supervisor.

What is the best response to the review findings?

A) Continue the current plan unchanged because six weeks is too early to use outcome data

B) Discharge the client because the stable scores show that counseling is ineffective

C) Explore the mismatch with the client and revise the goals or approach while continuing to monitor outcomes

---

Question 136

> A 15-year-old is brought to an outpatient clinic by a legal guardian. Clinic policy requires guardian consent for routine counseling and calls for the adolescent's assent when developmentally appropriate. The adolescent appears reluctant but is able to understand a plain-language description of services.

How should the counselor proceed with the intake?

A) Obtain only the guardian's signature because assent has no relevance when a minor cannot legally consent alone

B) Obtain the guardian's consent and invite the adolescent's informed assent, including discussion of privacy and its limits

C) Ask the adolescent to sign the adult consent form and exclude the guardian from all intake decisions

---

Question 137

A licensed counselor is asked to use a specialized intervention for which the counselor lacks training and supervised experience. Which response best meets professional accountability duties?

A) Use the intervention after reading the manufacturer's overview because licensure authorizes all counseling methods

B) Ask the client to waive concerns about the counselor's limited experience before beginning the intervention

C) Obtain appropriate training and supervision or refer the client to a competent provider

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Question 138

When the main purpose of an initial screening tool is to minimize missed cases of a potentially serious condition, which characteristic is most desirable?

A) High interrater reliability

B) High sensitivity

C) High specificity

Question 139

> While reviewing a strained caseload, a counselor realizes that a scheduling error delayed follow-up with a client who recently reported passive suicidal thoughts and completed a safety plan. The counselor does not know the client's current status.

What should the counselor do?

A) Promptly reassess or arrange appropriate contact and coverage, disclose the lapse in supervision, and address the caseload conditions that contributed

B) Begin a personal wellness routine and wait for the client to initiate contact if additional support is needed

C) Transfer the entire caseload immediately because one follow-up error proves the counselor is unfit to practice

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Question 140

At intake, a client reports receiving recent treatment from a primary care clinic and asks the counselor to speak with the medical provider. The counselor believes medication and laboratory information may be relevant to assessment.

What is the most appropriate way to obtain the collateral information?

A) Call the provider immediately because the client's verbal request authorizes disclosure of any medical information

B) Ask the client's relative to obtain the records so formal permission is unnecessary

C) Discuss the specific information and purpose with the client, obtain appropriate authorization, and request only relevant records

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Question 141

A 46-year-old client seeking help for “panic” describes a first episode of chest pressure, shortness of breath, and dizziness that began while climbing stairs. The client has diabetes and has never received a medical evaluation for these symptoms.

What is the best assessment decision?

A) Confirm panic disorder because the symptoms occurred suddenly and were frightening

B) Administer only an anxiety inventory before considering any other explanation

C) Encourage prompt medical evaluation while continuing to assess the psychological features and context of the episode

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Question 142

In family counseling, the parents say major decisions traditionally include a respected grandparent. Their 16-year-old asks to discuss some concerns without that grandparent present. All family members are willing to discuss how sessions will be structured.

How should the counselor proceed most responsively?

A) Exclude the grandparent because adolescent privacy should always override family decision-making traditions.

B) Include the grandparent in every session because the parents have defined the family’s cultural practice.

C) Explore what the grandparent’s involvement and the adolescent’s privacy mean to each person, then negotiate roles and session boundaries.

Question 143

A client in stable outpatient treatment will relocate to another state in six weeks. The client wants continued counseling, has chosen a prospective provider, and authorizes limited coordination. The current counselor cannot practice in the destination state.

Which plan best supports the transition?

A) Coordinate the authorized handoff, confirm interim and emergency supports, and plan the ending before the move

B) Continue telehealth after the move until the client feels established, regardless of practice authority

C) End services now so the client has enough time to locate care without influence from the current counselor

Question 144

An outpatient practice is considering an artificial intelligence service that creates draft progress notes from recorded sessions. A counselor has not previously studied the professional responsibilities associated with such tools.

Which learning activity would best prepare the counselor to participate in the practice's decision?

A) Complete credible ethics and legal training addressing consent, privacy, data handling, documentation, and applicable jurisdictional requirements

B) Rely on the vendor's product demonstration because it explains the tool's security and accuracy features

C) Delay learning about the tool until the practice adopts it and a client raises a concern

Question 145

During a fourth session, a client says, “You keep moving me to the next topic before I finish. I leave feeling unheard.” The counselor notices an urge to explain that session time is limited.

Which response would best address the interaction at this moment?

A) “I appreciate you telling me. I may have been moving too quickly; can we look at when you felt cut off and what you needed from me?”

B) “Our time is limited, so I sometimes have to redirect us to cover everything in your treatment plan.”

C) “Let’s set that concern aside for now and return to the situation you were describing at work.”

Question 146

A 19-year-old reports severe dietary restriction and fainting. During assessment, the client has a resting heart rate of 42 beats per minute, appears weak, and reports being unable to keep fluids down since yesterday.

Which disposition is most appropriate?

A) Begin standard weekly outpatient counseling because motivation for change is the main level-of-care criterion

B) Arrange immediate medical evaluation because the findings suggest possible medical instability

C) Defer all action until a body image inventory has been scored

Question 147

A counselor is competent in a specialized treatment that a prospective client needs, but the client requires twice-weekly sessions for the next month. The counselor has no openings beyond one appointment every other week.

Which response best reflects an appropriate evaluation of fit?

A) Accept the client because competence with the treatment is more important than session availability

B) Reserve the counselor's infrequent openings and ask the client to wait for the schedule to improve

C) Explain the availability limitation and help the client consider qualified providers who can offer the needed frequency

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Question 148

Which activity best supports continuing competence after initial training in an evidence-supported approach?

A) Repeat the same protocol without modification so fidelity is never affected by client context

B) Count the number of clients treated because experience alone demonstrates continued competence

C) Use deliberate practice, qualified feedback, relevant outcome information, and current evidence to refine implementation

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Question 149

At the end of counseling, a client offers the counselor a small handmade item that has cultural significance as an expression of gratitude. The counselor feels unusually pleased by the client's admiration and worries that either accepting or declining could affect the client.

What is the most self-aware course of action?

A) Accept the item immediately because declining a culturally meaningful gift would be disrespectful

B) Reflect on the counselor's reaction, explore the gift's meaning with the client, and use consultation and relevant policy to guide a transparent response

C) Decline the item immediately because the power difference makes every client gift inappropriate

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Question 150

A client referred for mild depressive symptoms says the most urgent concern is keeping housing after work hours were reduced. The counselor believes increasing pleasant activities could help mood, while the client wants short-term planning around housing stability first.

How should the counselor establish the initial focus?

A) Begin behavioral activation because the diagnosis should determine the first treatment target

B) Address only the housing concern and exclude depressive symptoms from the plan

C) Collaboratively prioritize housing stability while connecting mood support to the client's immediate needs and preferences

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Question 151

A college counselor who completed an introductory workshop on dialectical behavior therapy wants to start a skills group. The counselor has never led a therapy group or received feedback on group facilitation.

What is the best professional development decision?

A) Start the group because teaching skills requires less competence than providing group counseling

B) Refer all interested students elsewhere because competence in group work cannot be developed on the job

C) Obtain group-specific training and supervision and, if feasible, cofacilitate with a competent group counselor

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Question 152

A counselor is licensed in the designated home state of a counseling compact. A long-term client relocates to a compact member state whose rules require the counselor to hold an active privilege to practice there. The counselor's privilege application is still pending, and no emergency exception applies.

What should the counselor do?

A) Continue telehealth because a home-state license is automatically valid in every member state

B) Continue for 30 days because an established counseling relationship predates the relocation

C) Refrain from practicing with the client in that state until the privilege is active and arrange lawful continuity of care

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Question 153

Janelle wants to return to work after cancer treatment and has authorized collaboration with a vocational rehabilitation specialist. The specialist recommends an immediate full-time return, while Janelle tells the counselor that a graded schedule feels sustainable.

How should the counselor contribute to coordination?

A) Endorse the specialist's plan because vocational decisions should be determined by the specialist

B) Communicate Janelle's stated pacing goal and facilitate a plan that addresses functional information and available supports

C) Withhold Janelle's preference so the specialist can make an unbiased recommendation

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Question 154

During an initial interview, a client repeatedly looks toward the office door, speaks quietly, and pauses before answering. The counselor is documenting the mental status examination.

Which note best separates observation from interpretation?

A) “Client looked toward the door repeatedly and spoke softly; the reason for this behavior was not established.”

B) “Client was paranoid and expected an attacker to enter the office.”

C) “Client's guarded behavior confirmed a psychotic thought process.”

Question 155

A counselor learns that surgery will require a six-week medical leave beginning next month. Several clients are in active treatment, and the agency has clinicians who can provide temporary coverage.

Which continuity plan is most appropriate?

A) Wait until the final session before leave to avoid creating unnecessary concern

B) Give all clients the covering clinicians' names and transfer every case without discussion

C) Discuss the interruption in advance, arrange individualized coverage or alternatives, and provide emergency contact procedures

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Question 156

A community counseling client has just received an eviction notice. The client is pacing, speaking loudly, and saying, “No one will help me, so stop asking questions.” The counselor has already established that the client has no weapon and is not threatening self-harm or harm to others.

Which in-session response is most likely to reduce escalation?

A) Present a detailed list of housing programs so the client can immediately choose among them.

B) Use a calm voice, acknowledge the client’s overwhelm, and offer a choice between sitting or continuing to walk while talking.

C) Explain that productive counseling requires the client to lower their voice before the session can continue.

Question 157

What is the best foundation for an initial treatment plan?

A) A synthesis of assessment findings, diagnosis when applicable, client priorities, strengths, barriers, and context

B) A standard protocol selected solely from the diagnostic label before discussing the client's preferences

C) A list of every problem identified by the counselor, addressed simultaneously in order of clinical complexity

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Question 158

Mr. Chen, an older adult receiving grief counseling, has authorized his daughter to coordinate transportation and appointment times only. His daughter calls to arrange a ride and asks whether he talks about his grief during sessions.

How should the counselor respond?

A) Coordinate the ride details only and invite Mr. Chen to decide whether he wants to expand the information-sharing scope

B) Confirm whether grief is discussed but avoid giving any details about what Mr. Chen says

C) Send the daughter the latest note because her transportation support makes her part of the care team

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Question 159

A client sends detailed trauma information to a counselor's personal email address. The practice prohibits clinical communication through personal accounts and provides a secure client portal. The message does not indicate an emergency.

What is the counselor's best response?

A) Move further communication to the approved system, explain the privacy limit, and document the incident according to policy

B) Continue the email exchange because the client initiated communication through the personal account

C) Forward the message to the counselor's personal cloud storage so it will be available for later documentation

Question 160

In an integrated care clinic, a client with depression and diabetes has authorized the counselor to coordinate with the primary care provider. The provider asks whether the client is "compliant." The client set a goal of walking for 10 minutes three days per week and completed five of the six planned walks during the past two weeks, while reporting that foot pain was the main barrier.

What is the counselor's best response to the provider?

A) Share the concrete behavioral progress, the reported barrier, and the client's stated goal without using a global label

B) Describe the client as noncompliant because the walking plan was not completed exactly as prescribed

C) Send the full psychotherapy note so the provider can decide how motivated the client is

Question 161

An adult client returning to the community after incarceration has authorized coordination with a housing coordinator and a workforce coach. The client has a promising job interview in six days but lacks the identification documents required for hiring.

Which coordination step best supports the client's immediate goal?

A) Tell the prospective employer about the client's history so the employer can prepare for possible delays

B) Reserve the next counseling session for emotional reactions because document replacement is outside counseling

C) Arrange a warm handoff to an identification-replacement resource and align the coordinator's and coach's tasks with the interview timeline

Question 162

A client at an outpatient intake reports drinking heavily every day for several months and having a previous withdrawal seizure. The client stopped drinking yesterday and now has marked tremor, sweating, nausea, and increasing agitation.

Which level-of-care decision is most appropriate?

A) Begin routine weekly counseling and reassess withdrawal symptoms at the next visit

B) Refer for a nonurgent substance-use evaluation within the next several weeks

C) Facilitate immediate medical assessment for potentially dangerous alcohol withdrawal

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Question 163

A counselor is preparing to meet a transferred client. The most recent notes use the client's chosen name, Luca, and they/them pronouns, while an older intake form lists a different name and pronouns. The transfer summary identifies current goals but does not explain the discrepancy.

How should the counselor prepare for the first session?

A) Copy the older demographic information into the agenda because the original intake is authoritative

B) Avoid using any name or pronouns unless the client raises the subject first

C) Use the latest client-stated information, note the discrepancy for respectful confirmation, and review the current goals

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Question 164

A client with depression agreed to complete five behavioral activation activities but completed none. The client says, “The list looked impossible when I got home, so I stayed in bed.”

How should the counselor respond when continuing this intervention?

A) Keep the same assignment for another week so the client has enough time to demonstrate commitment.

B) Replace behavioral activation with cognitive restructuring because noncompletion shows the method is ineffective.

C) Explore the barriers and collaboratively choose one small, personally meaningful activity with a specific time.

Question 165

A counselor who works with children exposed to violence has begun having intrusive images from clients' stories, feeling emotionally numb at home, and dreading sessions that previously felt manageable. The counselor is still meeting basic job duties.

What is the most appropriate response to these changes?

A) Continue the current caseload until job performance clearly declines

B) Avoid discussing difficult cases outside sessions so the images receive less attention

C) Seek supervision or consultation, assess the impact on care, and develop supports that may include caseload adjustments

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Question 166

During an imagery exercise related to a traumatic memory, a client becomes still, stops responding to the counselor’s questions, and appears disoriented. The client had previously agreed on a signal for pausing the exercise.

What should the counselor do now?

A) Continue the imagery at a slower pace so the client can complete the therapeutic task.

B) Ask the client to interpret the meaning of becoming still before changing the intervention.

C) Stop the imagery and use the agreed pause procedure to support present-moment orientation.

Question 167

A Deaf client who uses American Sign Language begins counseling for adjustment-related stress. The client prefers a qualified interpreter and visual summaries of between-session tasks. The agency can provide both without changing the frequency of care.

How should the counselor incorporate this information into the treatment plan?

A) Replace the client's stated goals with communication goals before addressing the presenting concern

B) Use written communication only so that no third party is present during counseling

C) Document the communication accommodations and apply them while pursuing the client's chosen goals

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Question 168

A college student asks a counselor to sign a disability-accommodation form certifying a specific functional limitation. The counselor has not assessed that limitation, and the college requires the signer to have a reasonable clinical basis for each certification.

What should the counselor do?

A) Decline to certify an unsupported limitation and offer an appropriate assessment or referral process

B) Sign the form because supporting access is more important than documenting a clinical basis

C) Ask the student to write the limitation and sign it without reviewing the stated basis

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Question 169

Which counselor statement is the clearest example of immediacy?

A) “You felt dismissed when your supervisor changed the subject yesterday.”

B) “What possibilities have you considered for responding to your supervisor?”

C) “As we discuss this, I notice you look away each time I offer an observation. What is happening between us right now?”

Question 170

Which statement most accurately describes professional consultation?

A) It transfers responsibility for the case to a specialist who directs all subsequent services

B) It provides focused expert input while the counselor retains responsibility for professional decisions

C) It is interchangeable with clinical supervision because both relationships are necessarily evaluative

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PART B — ANSWER KEY AND EXPLANATIONS

Answers correspond to the question numbers in Part A.

Question 1 — Correct: C

Correct Answer: Clarify the conflict through appropriate consultation, comply with controlling law, and minimize ethical harm

Explanation: A counselor should determine how the law actually applies and use consultation to identify the most ethically responsible lawful course. Neither ethics nor law should be dismissed automatically; the counselor remains responsible for reducing avoidable harm within legal constraints.

Question 2 — Correct: B

Correct Answer: Regularly verify eligibility, services, accessibility, languages, costs, wait times, and contact routes

Explanation: A referral directory is useful only when its entries reflect the practical conditions clients will encounter. Regular verification reduces failed referrals caused by changed eligibility, capacity, cost, or access. Popularity ratings do not establish fit or current availability.

Question 3 — Correct: C

Correct Answer: The client reported missing three shifts; the counselor observed slowed speech and tearfulness during the discussion.

Explanation: This statement identifies the source of the employment information and separately records behaviors the counselor directly observed. The other statements add unsupported conclusions about severity, motivation, or events outside the session. Neutral attribution makes the record more useful to later providers.

Question 4 — Correct: B

Correct Answer: It decreases because a larger proportion of positive results may be false positives

Explanation: When prevalence is low, even a useful screening test may yield a lower proportion of true positives among all positive results. Positive predictive value depends partly on prevalence and is not interchangeable with sensitivity.

Question 5 — Correct: B

Correct Answer: Arrange clinical supervision while obtaining structured training in the treatment

Explanation: The counselor has a defined competence gap in a specialized intervention, and ongoing supervision permits skills to be developed with oversight before independent delivery. A brief webinar or one-time peer opinion does not provide enough practice, feedback, or accountability for implementing the full protocol.

Question 6 — Correct: A

Correct Answer: The client will use two coping strategies during at least four work shifts each week for the next month

Explanation: This statement specifies an observable client behavior, frequency, setting, and time frame. The broad improvement statement is a goal, while the statement describing what the counselor will teach is an intervention.

Question 7 — Correct: B

Correct Answer: Emphasize stabilization and immediate supports before reconsidering intensive trauma processing

Explanation: Frequent dissociation, limited grounding capacity, and unsafe housing indicate that intensive trauma processing may currently destabilize the client. The plan can prioritize safety and stabilization without permanently excluding later trauma-focused work.

Question 8 — Correct: B

Correct Answer: Explain the evidence and purpose in accessible language, invite the client's perspective, and discuss cultural meaning

Explanation: Collaborative diagnosis review includes transparent reasoning and attention to the client's understanding and cultural context. Treating the label as culturally neutral or ending the discussion would undermine informed participation in treatment planning.

Question 9 — Correct: C

Correct Answer: Limit services to assessments within current competence and arrange consultation or referral to a qualified evaluator

Explanation: Experience with brief screeners does not transfer automatically to a comprehensive neuropsychological battery. The counselor should recognize the limit and involve a properly qualified evaluator rather than learning a high-stakes assessment solely from a manual or outsourcing only the final conclusion.

Question 10 — Correct: C

Correct Answer: Explain the counselor's duties and the expectation that members protect privacy, while clarifying that other members' behavior cannot be guaranteed

Explanation: Group informed consent should distinguish the counselor's confidentiality obligations from the conduct of other members. The counselor can establish expectations and discuss risks but cannot guarantee that every participant will preserve confidentiality.

Question 11 — Correct: A

Correct Answer: Arrange qualified language access, learn effective interpreter collaboration, and avoid presenting conversational skill as clinical fluency

Explanation: The client has identified an access method that supports accurate clinical communication, and the counselor can develop competence in working effectively with that resource. Family interpretation and categorical referral both shift the burden to the client instead of addressing access and the counselor's learning needs.

Question 12 — Correct: C

Correct Answer: Review the gains, plan a gradual ending if useful, and clarify booster and reentry options

Explanation: The agreed indicators support a termination discussion, while the client's concern can be addressed through a planned transition. Reviewing gains and access options supports autonomy and continuity without making routine weekly care indefinite.

Question 13 — Correct: C

Correct Answer: Explain the no-secrets policy, its limits, and how it applies to both partners, then obtain informed agreement

Explanation: The handling of individual disclosures is material to informed consent in couple counseling and should be explained before services begin. An absolute promise conflicts with the clinic's policy, while delaying the explanation prevents the partners from making an informed choice.

Question 14 — Correct: C

Correct Answer: Define a brief work interval, reinforce remaining in the area with drawing time, and record the behavior consistently

Explanation: This plan operationalizes the target behavior, adjusts the task demand, uses an identified reinforcer, and permits consistent monitoring. Vague expectations or inconsistent punishment make implementation and evaluation less reliable.

Question 15 — Correct: B

Correct Answer: Remove the unsupported title and describe the counselor's actual education, licenses, and credentials accurately

Explanation: A workshop does not become a certification simply because its title is similar. Correcting the website prevents a misleading claim and allows prospective clients to evaluate the counselor's genuine qualifications.

Question 16 — Correct: C

Correct Answer: Provide a qualified interpreter through an accessible, confidentiality-protective arrangement

Explanation: The scenario requires effective communication and states that the client does not consent to family involvement. A qualified interpreter meets the communication need while better protecting accuracy, autonomy, and confidentiality than an unwanted companion or an ineffective written exchange.

Question 17 — Correct: A

Correct Answer: Collaboratively establish paced, graded activity with planned rest before symptoms sharply increase.

Explanation: Pacing interrupts the boom-and-bust cycle by supporting sustainable participation rather than maximizing activity only when symptoms are low. Complete avoidance can reduce functioning, while overactivity on a good day is the pattern already contributing to prolonged setbacks.

Question 18 — Correct: A

Correct Answer: Prepare the rating scale, allow additional response time, and review the prior goal and fatigue pattern

Explanation: Reviewing the prior note allows the counselor to prepare an accessible format that Devon has already identified as useful. This preserves Devon's participation and connects the session to the established goal. Having a caregiver answer instead would unnecessarily replace Devon's voice.

Question 19 — Correct: A

Correct Answer: Explore the client's meaning of the symptoms, explain the purpose of screening, and use a suitable trauma measure alongside the interview

Explanation: The counselor should understand the client's own idiom of distress and combine culturally responsive interviewing with an appropriate screening tool. Neither prematurely assigning a diagnosis nor rejecting all standardized measures provides an adequate assessment.

Question 20 — Correct: A

Correct Answer: Complete the required verification and emergency checks before proceeding with clinical services

Explanation: Identity and current location are essential to privacy, jurisdiction, and emergency response in telehealth. Platform account information does not substitute for the practice's required checks, and postponing them could leave the counselor unable to respond lawfully or safely.

Question 21 — Correct: C

Correct Answer: Review the agreed criteria and remaining needs with the client, then decide on a taper or termination plan together

Explanation: The client has met the preidentified symptom and functioning targets, making a termination review appropriate. Readiness should be evaluated collaboratively, including any remaining needs and whether tapering would support the transition; neither zero anxiety nor abrupt termination is required.

Question 22 — Correct: B

Correct Answer: Facilitate prompt specialized and medical evaluation and participate in ongoing care only within demonstrated competence

Explanation: Fainting and rapid weight loss indicate a need for timely medical and eating-disorder expertise beyond general adolescent counseling. Coordinated referral addresses the limitation while still permitting a bounded role if continuity is useful and the counselor is competent to provide it.

Question 23 — Correct: A

Correct Answer: Activate the emergency protocol using the verified location and disclose only information needed for the response

Explanation: The client has reported an immediate lethal plan, and the connection failure does not eliminate the need for action. The preverified location permits the counselor to initiate the planned response while limiting disclosure to emergency needs.

Question 24 — Correct: A

Correct Answer: Respectfully interrupt, describe the participation pattern, and invite the group to examine how it is affecting them.

Explanation: The leader both blocks monopolizing and turns the observable pattern into therapeutic group work. A rule imposed without processing may create compliance, but it would miss members' current reactions and the opportunity for interpersonal learning.

Question 25 — Correct: C

Correct Answer: Offer a qualified interpreter, explain interpreter confidentiality and roles, and obtain the parent's agreement about communication

Explanation: A qualified interpreter supports accuracy, privacy, and appropriate boundaries in a sensitive family intake. Placing a child in the interpreter role can inhibit disclosure and create role strain, while reducing the interview to yes-or-no questions would impair assessment.

Question 26 — Correct: C

Correct Answer: Verify current requirements with authoritative jurisdictional and professional sources, then select training that addresses those requirements

Explanation: The counselor should anchor continuing education in current, authoritative information because telehealth requirements may vary and change. Course ratings and assumptions of nationwide uniformity do not establish that training is accurate for the jurisdictions involved.

Question 27 — Correct: C

Correct Answer: Study culturally responsive identification practices, examine referral criteria with the school team, and advocate for a transparent process

Explanation: This response combines relevant learning with a systems-level review of a process producing a disparity. Asking students to overcome an opaque system or offering remediation after exclusion leaves the referral mechanism unexamined.

Question 28 — Correct: B

Correct Answer: Track triggers, internal reactions, interventions, and client responses in a structured reflection and review patterns in supervision

Explanation: A structured record links the counselor's internal experience to observable clinical choices and outcomes, making recurring patterns easier to examine. Client reassurance would distort the counseling relationship, while avoidance would prevent learning and may unnecessarily limit practice.

Question 29 — Correct: C

Correct Answer: Bill only for the service actually delivered and documented, and explain the financial options honestly

Explanation: Claims must accurately reflect the service provided and the documentation supporting it. Altering a code or note to increase payment would be deceptive even if the client requested or benefited from it.

Question 30 — Correct: C

Correct Answer: The counselor will use role-play to help the client practice assertive requests during weekly sessions

Explanation: An intervention states what the counselor will do to help the client reach an objective. The frequency-reduction statement is a measurable objective, and the broad ability statement is a treatment goal.

Question 31 — Correct: B

Correct Answer: Teach one step verbally with a concrete example, practice it together, and use teach-back to check understanding.

Explanation: Breaking the skill into an accessible spoken step, practicing it, and checking learning adapts delivery without withholding the intervention. Reading a complex worksheet aloud preserves much of the original barrier, and waiting for ideal service conditions unnecessarily delays care.

Question 32 — Correct: B

Correct Answer: Review the prior goal, sleep-tracking plan, measure change, and questions needed to clarify what changed

Explanation: Good preparation integrates the prior plan and the new measure without assuming what the score change means. Clarifying questions can identify whether pain, sleep, mood, or another context influenced the result. Drafting a conclusion before speaking with Nia would turn preliminary data into an unsupported judgment.

Question 33 — Correct: A

Correct Answer: Explain the limitation, coordinate referral to a qualified trauma clinician, and offer interim support within the counselor's competence

Explanation: The requested treatment exceeds the counselor's current training, so transparent referral and continuity planning best fit the client's needs. A good alliance does not replace specialized competence, while an abrupt handoff could unnecessarily disrupt care.

Question 34 — Correct: A

Correct Answer: Give reasonable advance notice, discuss the change and available options, and update the financial agreement

Explanation: Advance notice and a revised agreement allow clients to make informed decisions and reduce disruption of care. Immediate or concealed implementation places the practice's financial interest ahead of transparent business conduct.

Question 35 — Correct: B

Correct Answer: Name the differing communication meanings and help the group develop norms that allow both engagement and respect.

Explanation: The counselor makes cultural and interpersonal meanings discussable without declaring one communication style superior. Collaborative norms can expand members' options while preserving safety; suppressing disagreement would also suppress an important source of group learning.

Question 36 — Correct: C

Correct Answer: How identity and current location will be confirmed, what to do after a connection failure, and how emergency support will be accessed

Explanation: Telemental health consent should address location verification, technology disruptions, and emergency procedures. A platform does not replace an agreed contingency plan, and current location may be essential when urgent help is needed.

Question 37 — Correct: B

Correct Answer: Help the student orient to the present, identify one immediate need, and take one manageable step at a time.

Explanation: Acute overwhelm calls for stabilization, reduced cognitive demand, and concrete, manageable action. A detailed retelling can increase arousal, while diagnostic education is unlikely to be useful until the student can process more information.

Question 38 — Correct: B

Correct Answer: Make a timely referral and share the pertinent clinical information authorized by the client

Explanation: The symptoms warrant prompt specialty evaluation, and the client's authorization permits focused coordination. A release does not make the entire record automatically necessary, and abruptly pausing all contact could create a gap before specialty care begins.

Question 39 — Correct: B

Correct Answer: Activate the continuity plan, notify clients through approved channels, and prioritize urgent coverage and secure record access

Explanation: The tested plan exists to maintain safe, confidential operations during exactly this kind of disruption. Waiting would leave urgent needs unmanaged, while public posting of client information would create an unnecessary privacy breach.

Question 40 — Correct: B

Correct Answer: Follow the jurisdiction's protective-action requirement, continue the safety assessment, and limit disclosures to what is necessary

Explanation: The stated facts support a serious, imminent, and identifiable threat under the supplied law. Reasonable protective action and limited disclosure meet the legal duty without the excessive exposure that a workplace-wide warning would create.

Question 41 — Correct: C

Correct Answer: Report the incident immediately, attempt appropriate mitigation, and follow the required assessment and notification process

Explanation: Misdirected protected information requires timely handling under the practice's stated incident procedure. Waiting or deleting evidence would impede mitigation, risk assessment, and accurate compliance documentation.

Question 42 — Correct: C

Correct Answer: Acknowledge the concern, invite the client to describe what understanding would look like, and discuss preferences and options

Explanation: This response manages defensiveness, centers the client's meaning, and makes room to evaluate fit collaboratively. Listing prior experience risks turning the exchange into a defense of the counselor and may imply that familiarity with some group members guarantees understanding of this client.

Question 43 — Correct: B

Correct Answer: Collaboratively build a hierarchy around interactions the client identifies as meaningful at family gatherings.

Explanation: Exposure is most relevant when it targets situations connected to the client's own goals and values. A collaboratively designed hierarchy can retain the effective mechanism of gradual exposure without imposing an individualistic goal of becoming broadly outgoing.

Question 44 — Correct: A

Correct Answer: Acknowledge the harm, identify what is and is not within the client's control, and rehearse a response the client chooses.

Explanation: This intervention validates the discriminatory context while building a concrete, client-directed coping and advocacy skill. Reframing the comments as a possible misunderstanding risks invalidation, and directing immediate formal action overrides the client's autonomy.

Question 45 — Correct: A

Correct Answer: Add a dated clarification attributing the statement to the brother and noting the client's disagreement

Explanation: The clarification corrects the ambiguity by identifying who supplied the information and preserving the client's differing account. This gives later readers a more accurate basis for clinical judgment without converting a disputed report into fact. Deleting the information would remove potentially relevant context rather than clarify it.

Question 46 — Correct: C

Correct Answer: Acknowledge the added frustration, offer to switch to the agreed telephone backup, and reestablish a manageable pace.

Explanation: The counselor addresses the technological source of escalation, validates its effect, and uses the previously established backup to preserve continuity. Requiring more repetition over an unstable connection would add burden, and ending the session prematurely would reduce support during distress.

Question 47 — Correct: B

Correct Answer: "You value punctuality, and I've noticed you have arrived late recently. How do you understand that difference?"

Explanation: Effective confrontation presents a discrepancy tentatively and invites the client to make meaning of it. Labeling the client as undependable is judgmental, while ignoring the pattern sacrifices potentially useful therapeutic information.

Question 48 — Correct: B

Correct Answer: Arrange a warm handoff with shared transition goals, confirmed appointments, and defined provider roles

Explanation: A coordinated step-down plan connects the adolescent to each needed service and clarifies responsibility across systems. A provider list alone leaves important gaps, while successful lower-level care may appropriately include ongoing school and family supports.

Question 49 — Correct: A

Correct Answer: Repeated symptom or behavior data, functioning indicators, and the client's view of progress

Explanation: Multiple sources gathered over time show both clinical change and its practical meaning to the client. Attendance alone, counselor impressions, or task completion cannot establish whether treatment is producing the intended outcomes.

Question 50 — Correct: C

Correct Answer: Sam folded their arms and was silent for about two minutes, then reported feeling angry and ignored after the schedule change.

Explanation: The correct entry records observable behavior, approximate duration, and Sam's own account without assigning a diagnosis or motive. The other choices convert a brief response into pejorative or unsupported conclusions. Neutral detail allows later readers to interpret the event in context.

Question 51 — Correct: C

Correct Answer: Remove unsupported content, verify the remaining information, and accept personal responsibility for the final entry

Explanation: Approval of a tool does not make its output clinically accurate or transfer authorship responsibility away from the counselor. Unsupported diagnosis and medication information must be removed, and every retained statement must be verified before entry into the record.

Question 52 — Correct: A

Correct Answer: Initiate emergency safety procedures and a higher level of intervention while following applicable protective duties

Explanation: The named target, stated intent, imminent timing, preparatory behavior, weapon access, and refusal of safety steps indicate an acute risk to another person. Routine follow-up or additional nonurgent testing would not adequately address the immediate danger.

Question 53 — Correct: B

Correct Answer: “I hear that my response missed the role of racism and left you feeling misunderstood. Would you tell me more about the impact?”

Explanation: The counselor names the missed context, takes responsibility for the impact, and invites the client’s perspective, all of which can support alliance repair. Claiming identical treatment for every conflict minimizes the client’s report and the sociocultural meaning of the event.

Question 54 — Correct: B

Correct Answer: Revise the activity objective collaboratively to remain measurable and attainable while addressing the same broader goal

Explanation: New physical information affects the feasibility of the original behavior but not the client's broader aim of increasing activity. A measurable alternative preserves the treatment direction and permits continued evaluation without requiring a diagnostic change.

Question 55 — Correct: A

Correct Answer: Rehearse noticing the physical cues, taking a timed pause, and composing a response only after arousal decreases.

Explanation: This intervention uses the client's identified warning signs and preferred active learning style to build a concrete interruption-and-response sequence. Insight exploration alone does not meet the immediate skill goal, and thought suppression is unlikely to produce reliable regulation.

Question 56 — Correct: B

Correct Answer: Decline to upload client data unless the tool completes the required review and receives practice approval

Explanation: The proposed use would expose identifiable clinical data and train an outside model before required safeguards are established. General treatment consent or a brief verbal disclosure does not replace the mandated privacy, security, contracting, and clinical reviews.

Question 57 — Correct: B

Correct Answer: A written plan linking early warning signs to coping steps, support contacts, and follow-up options

Explanation: The client has identified meaningful early indicators of worsening depression. Connecting those signs to specific actions and resources creates a practical maintenance and reentry plan rather than requiring severe deterioration before help is sought.

Question 58 — Correct: B

Correct Answer: Stop providing supervision under the expired credential and arrange qualified coverage while seeking reinstatement

Explanation: The state board expressly prohibits supervision under the expired credential. The active clinical license does not substitute for the required supervisor credential, and relabeling formal supervision as peer consultation would misrepresent the service.

Question 59 — Correct: A

Correct Answer: Transmit the requested supporting documentation through an approved secure method and record the disclosure

Explanation: The verified audit is authorized by the provider agreement and is limited to documentation supporting the claims. A secure, documented response meets the request without disclosing the client's entire record or refusing a permitted review.

Question 60 — Correct: A

Correct Answer: Current practice guidelines, peer-reviewed research, and training led by qualified professionals

Explanation: These resources allow the counselor to examine the evidence base, appropriate use, limitations, and skill requirements of an emerging intervention. Popularity, testimonials, sponsorship, and informal enthusiasm are weaker indicators of validity and professional competence.

Question 61 — Correct: B

Correct Answer: Avoid soliciting testimonials from current clients and use accurate, non-client-specific information in advertising

Explanation: Current clients may experience a request for a public review as pressure, and the review itself may reveal their relationship with the counselor. Truthful, non-client-specific advertising avoids exploiting that power difference and does not trade financial benefit for disclosure.

Question 62 — Correct: A

Correct Answer: A compliant agreement and access limited to the information the billing company needs to perform its authorized work

Explanation: An outside billing service may need protected information, but access should be governed by the required agreement and restricted by role and task. Payment involvement does not make unrestricted psychotherapy content necessary, while a total ban would prevent a permitted business function.

Question 63 — Correct: B

Correct Answer: Conduct a direct risk assessment, act on the clinical evidence, and document any decision that differs from the tool

Explanation: The tool is decision support, and its low-risk label conflicts with clinically significant evidence of a plan and means. The counselor retains responsibility for assessment and action and should document the basis for overriding the automated output.

Question 64 — Correct: A

Correct Answer: A qualified behavioral health ASL interpreter, while asking the client about the preferred communication setup

Explanation: Qualified interpretation supports accurate, direct communication while preserving the client's role in choosing how services are delivered. A spouse may introduce role conflicts, filtering, or inhibited disclosure and should not be assumed to be the client's preferred interpreter. Written communication alone is not an equivalent substitute unless the client selects it as appropriate.

Question 65 — Correct: C

Correct Answer: Provide access within the required period while preserving the original record and following the practice's access procedure

Explanation: Discomfort with informal shorthand is not a permitted basis for denial under the supplied rule. The counselor should preserve the original record and complete the access process rather than rewriting history or withholding the file.

Question 66 — Correct: B

Correct Answer: Pause the breathing exercise, shift to an external grounding method, and monitor the client's response.

Explanation: The client reports worsening symptoms, so the counselor should stop the aggravating technique, offer a lower-risk alternative, and assess its effect. Insisting on continued practice would disregard direct feedback and could further associate coping practice with loss of control.

Question 67 — Correct: A

Correct Answer: Promptly address workload and care quality with a supervisor, correct affected records, and establish sustainable supports and recovery practices

Explanation: The symptoms are already affecting documentation and professional functioning, so the response must address both current care quality and the conditions driving the strain. A distant vacation or longer work hours would not adequately manage present risks and may worsen impairment.

Question 68 — Correct: C

Correct Answer: Review confidentiality limits and help the members develop a mutually acceptable plan for incidental contact.

Explanation: The leader responds to a realistic boundary concern through informed discussion and collaborative planning. Automatic transfer is disproportionate to the facts, and requiring workplace details would add disclosure that is not necessary to address the concern.

Question 69 — Correct: B

Correct Answer: Acute stress disorder

Explanation: The trauma-related symptoms began after the crash and have persisted for 12 days, which falls within the duration window for acute stress disorder. Posttraumatic stress disorder requires symptoms lasting more than one month, while the described pattern is more specific than a general adjustment response.

Question 70 — Correct: B

Correct Answer: Whether reliability, validity, and norms are adequate for the intended language group and purpose

Explanation: Translation alone does not establish that scores have the intended meaning for a particular language group and use. Relevant evidence about reliability, validity, and normative interpretation is necessary before relying on the results.

Question 71 — Correct: C

Correct Answer: Explore the client's concern and collaborate on an acceptable practice, such as faith-consistent reflection or sensory grounding.

Explanation: Culturally responsive intervention begins with curiosity about the client's meaning and preserves choice while seeking a workable therapeutic process. Simply relabeling mindfulness as clinical dismisses the concern, whereas eliminating every attention-based option is unnecessarily rigid.

Question 72 — Correct: A

Correct Answer: Arrange a qualified interpreter and discuss communication preferences directly with the client

Explanation: Communication access should be guided by the client and supported through a qualified interpreter when requested. Relying on a relative can compromise accuracy, privacy, and role boundaries, while written notes may not match the client's preferred or most effective communication method.

Question 73 — Correct: B

Correct Answer: Remove the adolescent from the referee role and facilitate direct communication between the parents.

Explanation: Redirecting the parents to speak with each other interrupts triangulation and protects the adolescent from being recruited into the dispute. Asking the adolescent to compare the arguments would strengthen the very role the adolescent is resisting.

Question 74 — Correct: B

Correct Answer: Arrange prompt medical evaluation while assessing immediate safety and documenting the acute change

Explanation: An abrupt cognitive change following medication adjustments raises concern for a medical or medication-related condition and warrants prompt evaluation. A neurocognitive diagnosis should not be assigned from this brief presentation, and personality testing would not address the acute risk.

Question 75 — Correct: B

Correct Answer: Who the client or clients are, the counselor's role, how records and information will be handled, and the limits of confidentiality

Explanation: In family work, the parties need a shared understanding of who is receiving services and how information and records will be managed. The counselor should not assume that the scheduling parent is the sole client or accept an unstated custody-evaluation role.

Question 76 — Correct: B

Correct Answer: Center the client's employment goal, share the relevant transportation barrier, and define each participant's follow-up role

Explanation: A focused wraparound meeting should organize stakeholders around the client's stated outcome and the concrete barrier affecting it. Clear roles reduce duplication and make follow-through more likely. Exchanging all available information is broader than necessary and does not itself create an actionable plan.

Question 77 — Correct: C

Correct Answer: The client's score equals or exceeds the scores of about 84% of the norm group

Explanation: A percentile rank describes a score's relative position within the relevant norm group. It is not the percentage of items answered correctly and does not represent a number of standard deviations.

Question 78 — Correct: A

Correct Answer: Explore the client's own reasons for and against change while supporting autonomy.

Explanation: A client who sees consequences but remains uncertain is expressing ambivalence, so an autonomy-supportive exploration can strengthen self-motivated change. Premature action planning or argumentative challenge is likely to evoke resistance rather than resolve uncertainty.

Question 79 — Correct: A

Correct Answer: Address and look toward the client while allowing the interpreter to facilitate communication.

Explanation: The counselor should maintain the therapeutic relationship directly with the client while the interpreter supports communication access. Treating the interpreter as the primary conversational partner can marginalize the client and weaken engagement.

Question 80 — Correct: A

Correct Answer: Examine the possible countertransference in supervision and monitor how it is shaping interventions

Explanation: The counselor's personal loss appears to be influencing avoidance of clinically relevant material. Reflective supervision can help distinguish the client's needs from the counselor's reactions and support corrective changes without unnecessary self-disclosure or automatic termination.

Question 81 — Correct: A

Correct Answer: Acknowledge the significance of the invitation, explore what attendance would represent, and discuss an appropriate way to honor the milestone in counseling.

Explanation: The counselor maintains the stated boundary while treating the invitation as meaningful clinical material rather than merely rejecting it. Acceptance would conflict with the established role, while a brief refusal could overlook attachment, gratitude, and transition themes.

Question 82 — Correct: A

Correct Answer: Gather symptom and impairment data across settings while evaluating developmental, hearing, sleep, and other possible contributors

Explanation: Attention problems require evidence across contexts and careful consideration of alternative or co-occurring explanations. Sleep and hearing concerns are clinically relevant but neither automatically prove nor exclude attention-deficit/hyperactivity disorder.

Question 83 — Correct: B

Correct Answer: Complete skills-based training, obtain supervised feedback, and monitor client response while consulting current evidence

Explanation: Competence develops through knowledge, practice, feedback, and evaluation of client response, not through exposure to information alone. A manual or demonstrations can contribute to learning but do not by themselves establish the ability to apply an approach appropriately.

Question 84 — Correct: B

Correct Answer: A summary of functional barriers, current coping supports, and Marisol's stated attendance goal within the authorization

Explanation: Effective coordination shares information that is relevant to the client's goal and permitted by the authorization. Functional barriers and current supports can help the stakeholders plan access without exposing unrelated clinical history. Sending the full intake would disclose more information than the meeting requires.

Question 85 — Correct: C

Correct Answer: Stop the recording, address the unauthorized file under policy, and obtain specific consent before any future recording

Explanation: Consent to live telehealth does not satisfy the practice's separate written-consent requirement for recording. The counselor should stop further capture, manage the existing file through the approved incident process, and obtain valid consent before recording later sessions.

Question 86 — Correct: A

Correct Answer: Evaluate the temporal relationship to the medication and coordinate medical review before assigning a primary bipolar diagnosis

Explanation: The close onset after corticosteroid exposure makes a medication-related explanation important in the differential diagnosis. The available facts are insufficient to assume a primary bipolar disorder, and the presentation includes more than isolated insomnia.

Question 87 — Correct: A

Correct Answer: Generalized anxiety disorder

Explanation: The client has excessive, difficult-to-control worry across multiple domains for longer than six months, with associated symptoms and significant distress. The pattern is not limited to a discrete stressor or recurrent unexpected panic attacks.

Question 88 — Correct: A

Correct Answer: Identify warning signs, plan alternatives for payday, rehearse contacting the cousin, and specify what to do after a lapse

Explanation: An effective relapse-prevention plan links a known trigger to alternatives, social support, practiced coping, and a response to a lapse. Treating a lapse as total failure can intensify recurrence, and motivation alone is not a sufficient continuity strategy.

Question 89 — Correct: A

Correct Answer: When clarifying a specific fact after the client has had room to describe the experience.

Explanation: Closed questions are useful for obtaining or confirming focused information once adequate space has been provided for the client’s account. Open exploration and empathic reflection are generally better suited to discovering meaning and responding to emotional complexity.

Question 90 — Correct: B

Correct Answer: Privacy and technology risks, service limits, backup communication steps, and emergency procedures

Explanation: Telehealth consent should prepare clients for foreseeable privacy, technology, continuity, and emergency issues. Absolute security promises are misleading, and a counselor cannot disregard the laws tied to the client's location.

Question 91 — Correct: A

Correct Answer: Clarify the voluntary nature of counseling, the limited attendance disclosure, and the student's options before seeking consent

Explanation: The student cannot make an informed choice while holding inaccurate beliefs about coercion and disclosure. Correcting those beliefs before consent respects the actual referral terms and avoids implying that the entire record is open to the conduct office.

Question 92 — Correct: A

Correct Answer: Examine differences across settings and informants, obtain developmental and school data, and avoid drawing a conclusion from one reporter

Explanation: Discrepant reports may reflect real context differences, different expectations, or measurement limitations and should be investigated rather than erased. Diagnosis should integrate multiple sources and evidence of symptoms and impairment across relevant settings.

Question 93 — Correct: B

Correct Answer: What information, if any, may be shared, the limits of confidentiality, and the employee's choices about participation

Explanation: Referral by an employer does not by itself make the supervisor the client or authorize release of the full counseling record. Clarifying disclosure boundaries and participation expectations corrects the employee's misunderstanding before consent.

Question 94 — Correct: C

Correct Answer: Pause the advice and invite Malik to describe what happened for him as the group responded.

Explanation: The leader uses the here-and-now process by interrupting an unhelpful pattern and returning attention to Malik’s immediate experience. More advice or a quick topic shift would bypass the withdrawal that is already occurring in the room.

Question 95 — Correct: B

Correct Answer: Vicarious trauma can alter beliefs and worldview through empathic exposure, whereas burnout more often reflects chronic occupational strain and exhaustion

Explanation: Vicarious trauma is associated with cumulative empathic engagement with others' trauma and may affect cognitive schemas and worldview. Burnout is more commonly characterized by exhaustion, cynicism, and reduced effectiveness related to chronic work stress; neither distinction depends on the inaccurate absolutes in the other options.

Question 96 — Correct: A

Correct Answer: Remain quietly attentive and allow the client space to continue when ready.

Explanation: The client is emotionally engaged, oriented, and beginning to regulate, so purposeful silence communicates presence without interrupting the grief process. Rapid questions or premature reassurance could shift the counselor away from the client's experience.

Question 97 — Correct: A

Correct Answer: Enter a current, clearly labeled addendum that supplies the omitted information without changing the original date

Explanation: A transparent addendum improves clinical completeness while preserving the chronology and integrity of the signed note. Backdating an altered entry would make it appear that the information was recorded when it was not.

Question 98 — Correct: A

Correct Answer: Collaboratively adjust the objective and supports to fit the new caregiving demands while preserving the valued goal

Explanation: A major contextual change has made the original schedule unrealistic, not the broader goal irrelevant. Revising the objective with the client keeps the plan attainable and responsive while maintaining direction.

Question 99 — Correct: A

Correct Answer: Review the request, explain a supported denial in writing, and offer to add the client's statement of disagreement

Explanation: A right to request amendment does not require deletion of information the provider determines is accurate and complete. The supplied policy calls for a reasoned written response and a way to preserve the client's disagreement rather than erasing the clinical record or refusing the process.

Question 100 — Correct: A

Correct Answer: Recognize the reinforcement of the boundary drift, seek supervision, and restore a clinically appropriate structure

Explanation: The counselor's gratification is contributing to unequal and potentially dependency-reinforcing exceptions. Supervision and restored structure address both the counselor's reaction and the relational dynamic without shaming the client or treating idealization as inherently beneficial.

Question 101 — Correct: A

Correct Answer: Develop a graded sequence of meeting-related speaking tasks and track distress and participation

Explanation: A graded, measurable plan directly addresses the avoidance maintaining social anxiety and the client's functional workplace goal. A diffuse focus on self-esteem or an unrelated unstructured activity is less closely tied to the assessment findings and desired outcome.

Question 102 — Correct: A

Correct Answer: Confirm Aisha's priorities, assign nonduplicative responsibilities and deadlines, and document the follow-up plan

Explanation: The team needs a client-centered division of labor that closes the identified gap and prevents further duplication. Named responsibilities and deadlines make follow-through observable across organizations. Having everyone repeat the same tasks wastes effort and still leaves the landlord-document need unassigned.

Question 103 — Correct: B

Correct Answer: Advocate for an accessible service option while seeking training and consultation on disability-responsive practice

Explanation: The client's preference and the physical barrier call for advocacy rather than treating telehealth as an automatic substitute. Training and consultation can also strengthen the counselor's ability to provide responsive care without assuming that disability itself requires referral.

Question 104 — Correct: A

Correct Answer: Identify the competence gap and obtain appropriate training and consultation or refer that assessment component before using the instrument

Explanation: Competence with brief screeners does not establish competence with a complex personality inventory. Standardized directions and apparently straightforward cases do not remove the need for training in selection, administration, scoring, and interpretation.

Question 105 — Correct: C

Correct Answer: Confirm appropriate tail or prior-acts arrangements with the insurer and retain the coverage documentation

Explanation: A claims-made policy may leave a gap when a later claim concerns services performed before the policy ended. Confirming extended reporting or compatible prior-acts coverage before cancellation is a prudent professional accountability step.

Question 106 — Correct: A

Correct Answer: Designating a qualified professional to secure records and notify clients about care options

Explanation: A professional will prepares for unexpected interruption by assigning appropriate responsibility for record security and client notification or referral options. Family access or automatic destruction could compromise confidentiality and prevent orderly continuity of care.

Question 107 — Correct: C

Correct Answer: Judgment

Explanation: The hypothetical asks how the client would respond to a practical situation and is commonly used to sample judgment. It does not primarily test recall of past events or breadth of acquired factual knowledge.

Question 108 — Correct: A

Correct Answer: Help the client choose the request, role-play the conversation, and refine wording based on the client's feedback.

Explanation: Collaborative rehearsal builds skill, confidence, and ownership while keeping the client in control of the advocacy goal. Speaking for the client or imposing a fixed script would reduce autonomy and may not fit the client's voice or workplace context.

Question 109 — Correct: B

Correct Answer: Bring both data points into the session, ask about their context, and use the clarification to guide and document the current assessment

Explanation: A measure score and an earlier expectation can both be accurate within different time frames or contexts. Preparing to explore the discrepancy avoids prematurely labeling it and supports an updated assessment based on the client's explanation. Altering the prior note would erase what was known at that earlier time.

Question 110 — Correct: C

Correct Answer: Use simple truthful language, describe current safety routines, practice a coping skill, and check what the child understood.

Explanation: Developmentally appropriate facts, predictable safety information, and practiced coping support a child's sense of security without overwhelming the child. Checking understanding also lets the adult correct frightening misconceptions rather than assuming the message was received.

Question 111 — Correct: A

Correct Answer: Explain the body's alarm response in plain language, then rehearse a brief grounding strategy and ask the student to describe its effect.

Explanation: This approach offers understandable information, provides immediate skills practice, and checks the intervention's effect. A handout alone does not establish comprehension or skill use, and telling the student to ignore sensations may feel invalidating and does not teach regulation.

Question 112 — Correct: C

Correct Answer: Document the timing and effects as Leila reported them and contact the prescriber to relay the concern

Explanation: The counselor should accurately attribute the possible medication effects to Leila and communicate the concern to the prescriber under the existing authorization. The prescriber can assess whether the medication contributed and determine any change. Recommending a dose reduction would exceed the counselor's role.

Question 113 — Correct: B

Correct Answer: Structure equal speaking turns and reflect the pursue-withdraw pattern occurring between the partners.

Explanation: Couples counseling is better served by slowing the exchange and making the interactional cycle visible than by adjudicating whose memory is correct. Written accounts would continue the fact-finding contest rather than change the pattern maintaining the conflict.

Question 114 — Correct: B

Correct Answer: Preserve the record, notify the client when permitted, and obtain legal guidance about asserting privilege or responding

Explanation: The subpoena must be addressed, but the facts state that it does not itself defeat privilege. Preserving the record and seeking legal review protects confidentiality while allowing a lawful response; unilateral disclosure or destruction would be improper.

Question 115 — Correct: B

Correct Answer: Use a qualified translator, preserve the meaning in plain language, and provide a way to ask questions through qualified language support

Explanation: Qualified translation and a supported question process address both the written resource's accuracy and the client's ability to understand and use it. Bilingual ability alone does not establish skill in translating counseling content. Unreviewed machine output may distort clinically important meaning.

Question 116 — Correct: C

Correct Answer: Avoid conjoint work at this time and develop a safety-focused referral or individual service plan

Explanation: Ongoing intimidation and fear of retaliation make open participation in conjoint counseling unsafe and may increase risk. Communication training or session rules do not correct the coercive conditions described and should not substitute for safety-focused planning.

Question 117 — Correct: C

Correct Answer: Verify current eligibility, fees, scheduling, and accessibility without disclosing Priya's identity, then review the options and next steps with her

Explanation: Nine-month-old access information may no longer be reliable, and Priya's schedule and financial constraints are central to referral fit. Verifying program details without client-identifying information reduces the risk of a failed referral and preserves Priya's choice. Nothing in the scenario supports changing the level of care to inpatient treatment.

Question 118 — Correct: B

Correct Answer: Address the reported breach, assess its impact, and revisit confidentiality expectations and limits with the group

Explanation: The counselor cannot guarantee member behavior but remains responsible for responding to a reported violation and reinforcing informed group norms. Immediate dissolution is not automatically required, and a new guarantee would misstate the limits of group confidentiality.

Question 119 — Correct: B

Correct Answer: Collaborate on consistent sleep cues and stimulus-control practices that can flex with shifts and preserve valued family time.

Explanation: The core behavioral principles can be adapted to the client’s actual schedule and values rather than applied as a rigid clock time. This preserves both feasibility and cultural responsiveness; abandoning the approach entirely is not required by rotating shifts.

Question 120 — Correct: A

Correct Answer: Report the documentation accurately and request the board's instructions for resolving the deficiency

Explanation: Renewal and audit submissions must be truthful and supportable. Honest disclosure and board guidance preserve accountability, whereas recreating certificates or certifying undocumented compliance would misrepresent the record.

Question 121 — Correct: A

Correct Answer: Make the required report promptly and explain the confidentiality limit to the client in a supportive manner

Explanation: The supplied law uses reasonable suspicion, and the disclosure plus visible bruising satisfies that threshold without requiring the counselor to prove abuse. Investigating the caregiver before reporting could delay a legally required protective response.

Question 122 — Correct: C

Correct Answer: To integrate major themes, check shared understanding, and support continuity.

Explanation: A therapeutic summary organizes key material and gives the client a chance to confirm or correct the counselor’s understanding. It is not intended to introduce unprocessed interpretations or reproduce every detail of the session.

Question 123 — Correct: A

Correct Answer: Review the eligibility and access tradeoffs with Omar, then offer a warm connection to his preferred verified service

Explanation: A sound referral accounts for clinical fit and the client's real access constraints while preserving client choice. Both verified resources may be viable, so discussing tradeoffs and facilitating contact is more useful than simply supplying names. Reputation alone does not overcome incompatible hours, cost, transportation, or technology.

Question 124 — Correct: B

Correct Answer: Build concrete regulation and self-advocacy objectives around the client's sensory needs and communication preferences

Explanation: The plan should respond to the client's identified environmental triggers, preferred communication style, and goal of remaining employed. Requiring tolerance before considering accommodations or assuming job loss is necessary would disregard strengths and feasible contextual changes.

Question 125 — Correct: C

Correct Answer: Describe the observable change without assigning meaning and invite the client to clarify what is happening.

Explanation: A tentative observation respects the ambiguity of telehealth cues and opens space for the client’s own explanation. Assuming avoidance may be inaccurate, while ignoring a marked change could miss important emotional or technical information.

Question 126 — Correct: B

Correct Answer: Ask how, if at all, the client wants spiritual resources incorporated into goals and supports

Explanation: The counselor can recognize spirituality as a possible strength while allowing the client to define its relevance and form. Prescribing faith-community participation would make an unsupported assumption, whereas excluding spirituality would overlook a client-identified resource.

Question 127 — Correct: B

Correct Answer: Agree to target sleep first while noting how the other concerns will be monitored

Explanation: The client identifies sleep as the concern with the greatest immediate leverage and expresses readiness to work on it. Selecting that focus collaboratively while tracking the other problems respects client priorities without ignoring the broader case.

Question 128 — Correct: C

Correct Answer: Record Eli's exact report and the bottle labels, encourage prompt clarification from the prescriber or pharmacist, and coordinate under the authorization

Explanation: The counselor can document the discrepancy, support timely clarification, and relay relevant information without deciding which dose is correct. The prescriber or pharmacist is positioned to reconcile the medication instructions. Selecting a bottle or endorsing the alternating pattern would amount to medication guidance unsupported by verified prescribing information.

Question 129 — Correct: B

Correct Answer: Ask directly about suicidal thoughts, intent, plan, access to means, past behavior, and protective factors

Explanation: Nonsuicidal self-injury and a passive death wish require a direct, individualized suicide risk assessment even when the client denies suicidal intent for the cutting. The facts neither justify assuming no risk nor establish imminent danger requiring hospitalization without further assessment.

Question 130 — Correct: C

Correct Answer: Test-retest reliability

Explanation: Test-retest reliability estimates the consistency of scores across repeated administrations when the construct is expected to remain stable. Content validity concerns coverage of a construct, while interrater agreement concerns consistency between evaluators.

Question 131 — Correct: A

Correct Answer: Ask about the client's access preferences, explain the available service formats, and confirm that the selected format can meet the client's needs

Explanation: Effective intake identifies accessibility needs collaboratively and determines whether an available format is suitable. Automatically imposing the standard room or referring the client elsewhere would bypass the client's stated preferences and an available accommodation.

Question 132 — Correct: A

Correct Answer: Decline the custody evaluator role, clarify the limits of the treatment role, and provide only properly authorized factual information

Explanation: A treating counselor should not convert an existing therapeutic role into an impartial custody evaluation, especially without evaluating all relevant parties. Clarifying the role and limiting any release to authorized facts avoids an unsupported forensic opinion and a serious role conflict.

Question 133 — Correct: C

Correct Answer: Apply the governing laws and contracts, document the schedule, and use secure destruction after the required period

Explanation: Retention obligations can arise from law, regulation, contracts, and professional duties, so a practice needs a documented schedule that reconciles them. Automatic immediate destruction and indefinite retention both ignore those requirements and create avoidable risk.

Question 134 — Correct: B

Correct Answer: Verify the authorization and disclose only the information covered and reasonably needed for its stated purpose

Explanation: A valid authorization defines the permitted disclosure, but it does not justify sending unrelated clinical material. Verification and a purpose-limited release reduce unnecessary exposure while honoring the client's direction.

Question 135 — Correct: C

Correct Answer: Explore the mismatch with the client and revise the goals or approach while continuing to monitor outcomes

Explanation: Converging outcome and functioning data show little change, and the client identifies a plausible mismatch between the plan and the central problem. Collaborative revision uses the review findings constructively; unchanged scores do not by themselves require either automatic continuation or discharge.

Question 136 — Correct: B

Correct Answer: Obtain the guardian's consent and invite the adolescent's informed assent, including discussion of privacy and its limits

Explanation: The stated policy requires guardian consent and developmentally appropriate assent. Including the adolescent in a clear discussion of services and privacy supports meaningful participation without misrepresenting the guardian's role.

Question 137 — Correct: C

Correct Answer: Obtain appropriate training and supervision or refer the client to a competent provider

Explanation: Licensure alone does not establish competence in every specialized method. Training, supervision, or referral protects the client, whereas a waiver cannot transfer the counselor's duty to practice within competence.

Question 138 — Correct: B

Correct Answer: High sensitivity

Explanation: High sensitivity helps a screening tool identify people who have the condition and therefore reduces false negatives. Specificity instead concerns correctly identifying those who do not have the condition.

Question 139 — Correct: A

Correct Answer: Promptly reassess or arrange appropriate contact and coverage, disclose the lapse in supervision, and address the caseload conditions that contributed

Explanation: The unknown current status makes timely attention to the client's safety the immediate concern, while supervision and workload changes address professional functioning and recurrence. Wellness efforts alone do not resolve the missed follow-up, and one error does not automatically justify abandoning all clients.

Question 140 — Correct: C

Correct Answer: Discuss the specific information and purpose with the client, obtain appropriate authorization, and request only relevant records

Explanation: The counselor should obtain informed, appropriately scoped permission and limit the request to information relevant to the assessment purpose. A general verbal request or use of a relative does not provide the same clear authorization and control over disclosure.

Question 141 — Correct: C

Correct Answer: Encourage prompt medical evaluation while continuing to assess the psychological features and context of the episode

Explanation: New exertional chest symptoms in a client with a medical risk factor require medical causes to be evaluated. Psychological assessment can continue, but the counselor should not assume panic disorder before potentially serious contributors are addressed.

Question 142 — Correct: C

Correct Answer: Explore what the grandparent’s involvement and the adolescent’s privacy mean to each person, then negotiate roles and session boundaries.

Explanation: This response avoids assuming that either cultural tradition or individual privacy has a single fixed meaning for the family. Collaborative negotiation respects the adolescent’s voice and the family’s relational values without automatically privileging one over the other.

Question 143 — Correct: A

Correct Answer: Coordinate the authorized handoff, confirm interim and emergency supports, and plan the ending before the move

Explanation: A planned handoff within the client's authorization reduces gaps while respecting the current counselor's practice limits. Continuing without authority is inappropriate, and immediate termination would unnecessarily remove support before relocation.

Question 144 — Correct: A

Correct Answer: Complete credible ethics and legal training addressing consent, privacy, data handling, documentation, and applicable jurisdictional requirements

Explanation: The counselor needs independent, professionally grounded learning about the responsibilities implicated by an AI documentation tool. A vendor demonstration may inform product use, but it cannot substitute for education on ethical and jurisdiction-specific duties.

Question 145 — Correct: A

Correct Answer: “I appreciate you telling me. I may have been moving too quickly; can we look at when you felt cut off and what you needed from me?”

Explanation: This response accepts the feedback without defensiveness, acknowledges the counselor’s possible contribution, and invites specific information needed for repair. Explaining time limits before exploring the client’s experience would likely deepen the rupture.

Question 146 — Correct: B

Correct Answer: Arrange immediate medical evaluation because the findings suggest possible medical instability

Explanation: Severe restriction combined with syncope, marked bradycardia, weakness, and inability to retain fluids suggests possible medical instability. Assessment scores and routine outpatient scheduling should not delay urgent medical evaluation.

Question 147 — Correct: C

Correct Answer: Explain the availability limitation and help the client consider qualified providers who can offer the needed frequency

Explanation: Fit includes practical capacity to provide the intensity and continuity a client needs, not only technical competence. Transparent discussion and referral options protect access to timely care better than accepting a schedule that cannot meet the stated need.

Question 148 — Correct: C

Correct Answer: Use deliberate practice, qualified feedback, relevant outcome information, and current evidence to refine implementation

Explanation: Continuing competence requires active review of performance and evidence after initial training. Experience alone does not confirm quality, and rigidly ignoring client context can turn fidelity into mechanical or poorly fitted practice.

Question 149 — Correct: B

Correct Answer: Reflect on the counselor's reaction, explore the gift's meaning with the client, and use consultation and relevant policy to guide a transparent response

Explanation: The gift has relational and cultural meaning, and the counselor's pleasure in being admired may affect judgment. Reflection, dialogue, and consultation support a contextual response; automatic acceptance or refusal would bypass important factors.

Question 150 — Correct: C

Correct Answer: Collaboratively prioritize housing stability while connecting mood support to the client's immediate needs and preferences

Explanation: Housing instability is the client's urgent concern and a major context for mood and functioning. A collaborative plan can address that priority while retaining relevant depression support, rather than allowing either diagnosis or a single social need to exclude the other.

Question 151 — Correct: C

Correct Answer: Obtain group-specific training and supervision and, if feasible, cofacilitate with a competent group counselor

Explanation: Knowledge of a treatment model does not establish competence in group process and facilitation. Focused training, supervision, and supported practice address the identified gap while allowing competence to grow responsibly.

Question 152 — Correct: C

Correct Answer: Refrain from practicing with the client in that state until the privilege is active and arrange lawful continuity of care

Explanation: The receiving state's stated rule requires an active compact privilege, and a pending application does not satisfy it. An established relationship or home-state license does not create an exception that the scenario says is unavailable.

Question 153 — Correct: B

Correct Answer: Communicate Janelle's stated pacing goal and facilitate a plan that addresses functional information and available supports

Explanation: Collaboration should preserve Janelle's goal while helping the team integrate functional needs and available workplace supports. The specialist's expertise is relevant, but it does not replace the client's stated preference or the need for a coordinated plan. Withholding her preference would make the plan less client-centered and potentially less sustainable.

Question 154 — Correct: A

Correct Answer: “Client looked toward the door repeatedly and spoke softly; the reason for this behavior was not established.”

Explanation: This statement records directly observed behavior and appropriately acknowledges that its meaning has not been determined. The other choices convert limited observations into unsupported conclusions about paranoia or psychosis.

Question 155 — Correct: C

Correct Answer: Discuss the interruption in advance, arrange individualized coverage or alternatives, and provide emergency contact procedures

Explanation: Advance notice and individualized planning reduce the risk of an avoidable gap in care. Automatically transferring every client would not account for client preference or clinical need, while waiting until the last session would limit safe transition options.

Question 156 — Correct: B

Correct Answer: Use a calm voice, acknowledge the client's overwhelm, and offer a choice between sitting or continuing to walk while talking.

Explanation: Calm validation and limited choices can reduce demands, restore some control, and support de-escalation. Detailed problem solving is less usable while arousal is high, and requiring a quieter voice as a condition may intensify the power struggle.

Question 157 — Correct: A

Correct Answer: A synthesis of assessment findings, diagnosis when applicable, client priorities, strengths, barriers, and context

Explanation: An initial plan should integrate clinical findings with the client's goals and the factors likely to support or obstruct progress. Diagnosis alone cannot determine an individualized plan, and attempting to address every problem at once weakens prioritization.

Question 158 — Correct: A

Correct Answer: Coordinate the ride details only and invite Mr. Chen to decide whether he wants to expand the information-sharing scope

Explanation: The existing authorization permits logistical coordination but not discussion of session content. Staying within that scope supports the daughter's practical role while preserving Mr. Chen's control over any broader collaboration. Even confirming that grief is discussed would answer a clinical question outside the stated authorization.

Question 159 — Correct: A

Correct Answer: Move further communication to the approved system, explain the privacy limit, and document the incident according to policy

Explanation: The client's choice to use personal email does not eliminate the counselor's responsibility to follow secure communication policy. Redirecting the exchange, explaining the limitation, and documenting the event protects confidentiality without ignoring the client's message.

Question 160 — Correct: A

Correct Answer: Share the concrete behavioral progress, the reported barrier, and the client's stated goal without using a global label

Explanation: Specific behavioral data and the client's context give the provider usable information for coordinated care. A global term such as "noncompliant" obscures both substantial progress and the foot-pain barrier. The full psychotherapy note would include information unnecessary for the coordination question.

Question 161 — Correct: C

Correct Answer: Arrange a warm handoff to an identification-replacement resource and align the coordinator's and coach's tasks with the interview timeline

Explanation: The identification barrier directly threatens the client's near-term employment goal and is well suited to coordinated practical support. A warm handoff and aligned roles improve the likelihood of timely completion. Disclosing incarceration history to an employer is not necessary to solve the document problem.

Question 162 — Correct: C

Correct Answer: Facilitate immediate medical assessment for potentially dangerous alcohol withdrawal

Explanation: A prior withdrawal seizure plus current autonomic and neurologic symptoms indicates risk of severe withdrawal. Routine or delayed outpatient follow-up would not match the potential medical urgency.

Question 163 — Correct: C

Correct Answer: Use the latest client-stated information, note the discrepancy for respectful confirmation, and review the current goals

Explanation: The most recent client-stated name and pronouns are the best available information for respectful preparation. Flagging the discrepancy allows the counselor to confirm rather than assume while also maintaining continuity on current goals. Treating an older intake as automatically controlling could perpetuate outdated information.

Question 164 — Correct: C

Correct Answer: Explore the barriers and collaboratively choose one small, personally meaningful activity with a specific time.

Explanation: Behavioral activation is adjusted by reducing task difficulty, linking action to meaning, and making the step concrete. Reassigning an overwhelming list ignores the information gained from noncompletion, while one unsuccessful assignment does not establish that the whole approach is ineffective.

Question 165 — Correct: C

Correct Answer: Seek supervision or consultation, assess the impact on care, and develop supports that may include caseload adjustments

Explanation: Intrusive imagery, emotional numbing, and increased dread after repeated indirect trauma exposure are warning signs that warrant prompt attention even before basic duties fail. Supervision or consultation can help evaluate effects on clients and guide targeted supports rather than relying only on avoidance or endurance.

Question 166 — Correct: C

Correct Answer: Stop the imagery and use the agreed pause procedure to support present-moment orientation.

Explanation: The client’s sudden nonresponsiveness and disorientation indicate that the current exercise has exceeded the client’s capacity to remain engaged. Honoring the agreed pause and reorienting to the present is safer and more responsive than pressing forward or requesting interpretation during disconnection.

Question 167 — Correct: C

Correct Answer: Document the communication accommodations and apply them while pursuing the client's chosen goals

Explanation: A qualified interpreter and visual summaries make the plan accessible while preserving the client's autonomy and presenting priorities. Substituting communication goals or relying only on writing would disregard the client's stated needs and preferences.

Question 168 — Correct: A

Correct Answer: Decline to certify an unsupported limitation and offer an appropriate assessment or referral process

Explanation: Advocacy for access does not justify a clinical statement that lacks an assessment basis. An appropriate evaluation or referral supports the student's request through accurate documentation rather than an unsupported certification.

Question 169 — Correct: C

Correct Answer: “As we discuss this, I notice you look away each time I offer an observation. What is happening between us right now?”

Explanation: Immediacy focuses on the present interaction between counselor and client and invites exploration of what is occurring in the therapeutic relationship. The other statements address an outside event or future problem solving rather than the here-and-now relationship.

Question 170 — Correct: B

Correct Answer: It provides focused expert input while the counselor retains responsibility for professional decisions

Explanation: Consultation typically offers specialized guidance without creating the ongoing evaluative authority associated with clinical supervision. The counselor remains accountable for deciding how to use the consultant's input and for the services provided.
